Title Page

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Protocol Title: A Study of Remternetug Versus Placebo in Early Alzheimer's Disease Participants at Risk for Cognitive and Functional Decline

Protocol Number: J1G-MC-LAKI

Amendment Number: c

Compound: Remternetug (LY3372993)

Brief Title: A Study of Remternetug in Early Alzheimer's Disease

Study Phase: 3

Acronym: TRAILRUNNER-ALZ 3

Sponsor Name: Eli Lilly and Company

Legal Registered Address: Indianapolis, Indiana, USA 46285

Regulatory Agency Identifier Numbers:

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Approval Date: Protocol Amendment (c) Electronically Signed and Approved by Lilly on date provided below.

Document ID: VV-CLIN-144844

Medical monitor name and contact information will be provided separately.

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Protocol Amendment Summary of Changes Table

DOCUMENT HISTORY	
Document	**Date**
Amendment (b)	11 July 2025
Amendment (a)	04 December 2024
Original Protocol	28 June 2024

Amendment [c]

This amendment is considered to be nonsubstantial.

Overall rationale for the amendment

The main purpose of this amendment is to include Quick Dementia Rating System as a clinical outcome assessment.

Additional changes and a brief rationale are provided in the table below.

Section # and Name	Description of Change	Brief Rationale
1.3.3. Double-Blind Treatment Period	Updated the text above the table to read "For participants who discontinue the study during this period, see ETa visit in Section 1.3.4 of the double-blind observation period. An ETa visit should be conducted if a participant has received at least 1 dose of study drug. If dosing is interrupted due to ARIA or any other reason, continue to perform visits and other study procedures per the SoA"	To clarify that an ETa visit should be conducted upon early discontinuation and if a participant has received at least 1 dose of study drug.
	Added rows "QDRS – Participant" and "QDRS – Study Partner" at Visit 2, Visit 5, and Visit 8	To collect additional information regarding clinical status from the perspective of participants and study partners.
1.3.4. Double-Blind Observation Period	Added rows "QDRS – Participant" and "QDRS – Study Partner" at Visit 11 through Visit 15, EVa, EVb, and ETa	To provide additional information regarding clinical status from the perspective of participants and study partners.
5.2.1. Criteria for Open-Label Extension Period	Updated subsection "Open-label extension inclusion criteria" to read "Individuals are eligible to be included in the open-label extension period of the study only if they meet all entry criteria for the double-blind treatment period prior to enrollment and if all of the following criteria apply"	To provide clarity.

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Section # and Name	Description of Change	Brief Rationale
7.2. Participant Discontinuation/Withdrawal from the Study	Updated section to clarify that ETa and posttreatment follow-up visit should only be conducted if a participant has received at least 1 dose of study intervention. Removed "At the time of discontinuing from the study, if possible, the participant will complete procedures for an early discontinuation visit and, if the participant received at least 1 dose of study intervention, will also complete the posttreatment follow-up visit (V801), as shown in the SoA"	To provide clarity.
8.1.7. Quick Dementia Rating System	Added a new section for Quick Dementia Rating System	To align with the updates made in the SoA under clinical outcome assessments.
10.3.1. Definition of AE	In subsection "Events NOT meeting the AE definition," added "In line with clinical practice for this disease state, and based on the defined criteria for AEs and related clarifications (ICH 1994; FDA 2009; EMA 2017; CFR 2024), the following are not considered AEs:"	To provide clarity.
10.10. Abbreviations and Definitions	Added "QDRS"	Editorial consistency.
10.11 Protocol Amendment History	Added amendment(b) rationale and summary of changes	To update the amendment history.
11. References	Added the following citations CFR (2024) Duff et al. (2022) EMA (2017) FDA (2009) ICH (1994) Galvin (2015) Galvin et al. (2020)	Editorial consistency.
Throughout the document	Minor editorial and formatting changes	Editorial consistency.

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1. Protocol Summary

1.1. Synopsis

Protocol Title: A Study of Remternetug Versus Placebo in Early Alzheimer's Disease Participants at Risk for Cognitive and Functional Decline

Brief Title: A Study of Remternetug in Early Alzheimer's Disease

Regulatory Agency Identifier Numbers: IND: 137063; EU trial number: 2024-515656-20-00

Rationale: The pathophysiological process of Alzheimer's disease (AD) begins more than a decade before the clinical stage now recognized as AD dementia. Compounds targeting the underlying disease process may have greater benefit when started earlier in the disease continuum.

This Phase 3 study will evaluate the safety and efficacy of remternetug in participants who have early AD. The primary objective of Study J1G-MC-LAKI (LAKI) will assess whether treatment with remternetug delays the time to clinically meaningful progression as compared to placebo.

Objectives, Endpoints, and Estimands:

Objectives	Endpoints
Primary	
To assess the effect of remternetug versus placebo on time to clinically meaningful progression in participants with early AD	Time to clinically meaningful progression as measured by CDR
Secondary	
To assess the effect of remternetug versus placebo on other measures of clinical progression in participants with early AD	Change from baseline through the participant's final visit, for these clinical outcome assessments: ● CDR-SB ● MoCA ● Category Fluency ● CPAL ● ISLT ● iDSSTm ● CFI ● MBI-C ● ADCS-ADL-PI ● Cognitive composite score involving a combination of the individual assessments above may be analyzed. Additional details will be provided in the SAP.

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Objectives	Endpoints
To describe the safety of remternetug	Standard safety assessments, including ● Spontaneously reported AEs ● clinical laboratory tests ● vital signs MRI (ARIA and emergent radiological findings) Hypersensitivity reactions Injection site reactions C-SSRS
To assess peripheral PK and presence of anti-remternetug antibodies	PK of remternetug TE ADA against remternetug

Abbreviations: AD = Alzheimer's Disease; ADA = anti-drug antibodies; ADCS-ADL-PI = ADCS Activities of Daily Living – Prevention Instrument; AEs = adverse events; ARIA = amyloid-related imaging abnormalities; CDR = Clinical Dementia Rating; CDR-SB = CDR-sum of boxes; CFI = Cognitive Function Index; CPAL = Continuous Paired Associate Learning; C-SSRS = Columbia-Suicide Severity Rating Scale; iDSSTm = International Daily Symbol Substitution Test-Medicines; ISLT = International Shopping List Test; MBI-C = Mild Behavioral Impairment Checklist; MoCA = Montreal Cognitive Assessment; MRI = magnetic resonance imaging; PK = pharmacokinetic; SAP = statistical analysis plan; TE = treatment-emergent.

The primary clinical question of interest is: *What is the difference in time to clinically meaningful progression between participants with early AD who receive remternetug versus those who receive placebo, regardless of change in permitted concomitant medications and discontinuation of the study intervention other than discontinuation due to the use of a prohibited medication?*

Overall Design: Study LAKI is a multicenter, randomized, double-blind, placebo-controlled, parallel-group, Phase 3 study of remternetug for the treatment of participants with early AD. After screening, participants who meet the study entry criteria will be randomized in a 1:1 ratio to 1 of 2 treatment groups for the 78-week Double-Blind Treatment Period:

- Remternetug, given subcutaneously at a total of up to 15 time points:
  - 400 mg at randomization
  - 400 mg at 8 weeks after randomization
  - 800 mg at 16 weeks after randomization
  - 800 mg at 24 weeks after randomization
  - 800 mg at 32 weeks after randomization, and
  - 800 mg at every fourth week thereafter, up to and including 72 weeks after randomization.
- Placebo, given subcutaneously at matching time points.

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After the Double-Blind Treatment Period, participants will continue to have assessments in the Double-Blind Observation Period until the target number of events for the primary endpoint is achieved or until the study is ended.

At the sponsor's discretion, the study may include an Open-Label Extension Period. In this period, certain consenting participants who previously received placebo will receive remternetug at the same dose levels and frequencies described for the Double-Blind Treatment Period.

This study includes a follow-up visit for participants who received at least 1 dose of study intervention at the time of their early discontinuation from, or completion of, the study.

Brief Summary: This study measures the time to clinically meaningful progression in early AD. For this purpose, clinically meaningful progression is defined as an increase from baseline at two consecutive time points either in Clinical Dementia Rating sum of boxes score of 1 or greater or in Clinical Dementia Rating global score of 0.5 or greater. In this study, remternetug is compared to placebo. Both are given by injection under the skin. Study participants are persons with early AD.

The overall study duration depends on the length of time needed for events of clinical progression to occur. Thus, the study duration for each participant will vary. The maximum planned study duration of an individual is expected to be approximately up to 255 weeks. In addition, eligible individuals who received placebo during the Double-Blind Treatment Period may opt to extend their participation by an additional 78 weeks if they consent to receive remternetug in the Open-Label Extension Period.

The visit frequency depends on the study period. Generally, during the Double-Blind Treatment and Open-Label Extension Periods, the first 6 visits are about 8 weeks apart. Later visit intervals in these periods vary from about 6 to 12 weeks apart. In the Double-Blind Observation Period, visits are about 26 weeks apart. Throughout the study, all visits will have remote components.

Study Population: The study will enroll participants with early AD, which encompasses preclinical AD and MCI due to AD. Participants will have a Mini-Mental State Examination score ≥27 and a Functional Activities Questionnaire score <6.

Number of Participants: Up to approximately 1400 participants will be randomly assigned to study intervention.

Intervention Groups and Duration: See the "Overall Design" and "Brief Summary" sections of this synopsis.

Ethical Considerations of Benefit/Risk: Measures are taken to minimize risk to individuals participating in this study. The potential risks identified in association with remternetug treatment are justified by the anticipated benefits that may be afforded to the early AD population by removing amyloid plaque.

Data Monitoring Committee: Yes

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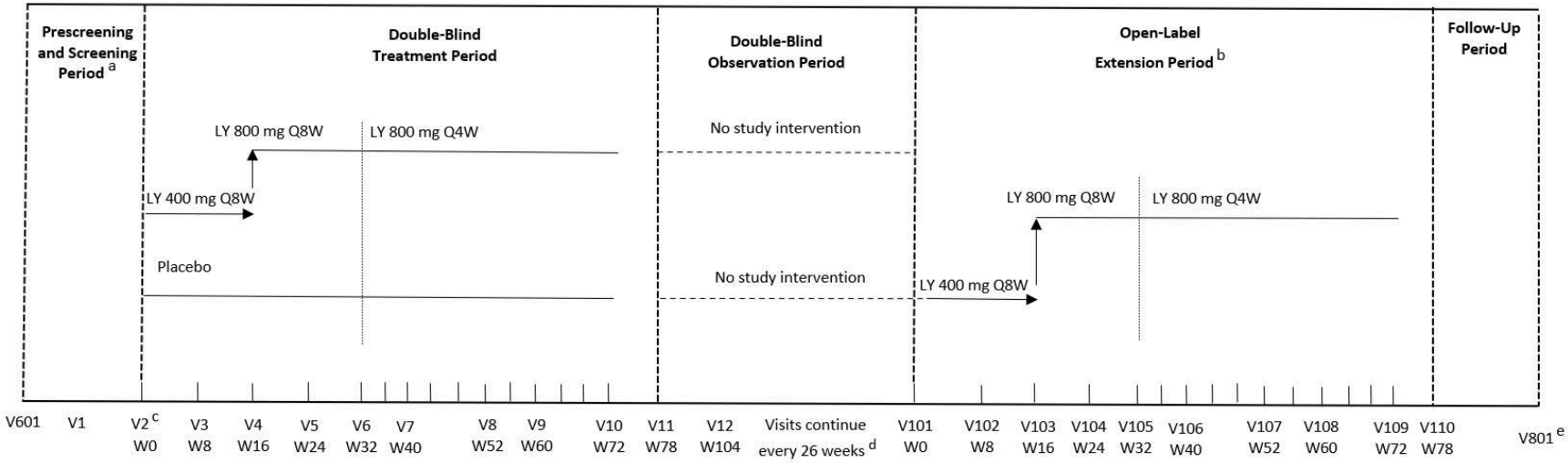
1.2. Schema



Abbreviations: LY = remternetug (LY3372993); Q4W = every 4 weeks; Q8W = every 8 weeks; V = visit; W = week.

a Visit 601 and Visit 1.

b If remternetug meets success factors defined by sponsor, then eligible participants who were randomly assigned to placebo and complete the Double-Blind Observation Period may have access to remternetug.

c Randomization (enrollment, that is, assignment to treatment) occurs at Visit 2.

d Double-Blind Observation Period continues until the minimum number of participants are enrolled and the target number of events for the primary endpoint is achieved, up to approximately 234 weeks.

e Visit 801 (follow-up) occurs at least 20 weeks after a participant's last dose of study intervention. If at the time of study completion or early discontinuation more than 20 weeks have passed since the participant's last dose of study intervention, Visit 801 is not required.

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1.3. Schedule of Activities (SoA)

Throughout the study, all visits will have remote components.

1.3.1. Prescreening Period

Prescreening Period		**Comment**
Visit number	**601**	Visit may occur over more than 1 day.
Timing	from signing ICF to availability of prescreening plasma P-tau result	
Visit interval tolerance (days)	—	
Informed consent of participant	X	Relevant ICF must be signed before any protocol-specific tests or procedures are performed.
Register visit with IWRS	X	
Inclusion/exclusion criteria; confirmation of eligibility	X	Inclusion and exclusion criteria should be reviewed as information becomes available.
Demographics	X	Includes, but is not limited to, ethnicity (where permissible), year of birth, sex, race, and years of education.
Adverse events (AEs)	X	Any events that occur after signing the ICF are considered AEs (Section 8.3). Additional data are collected for certain AEs (Section 8.3.3).
APOE sample collection	X	Sample will be tested for *APOE* if P-tau result is eligible. Samples may be stored and used to support the development of new diagnostic tools or assays. Stored samples may also be used to assess an individual's eligibility for future clinical trials if the individual consents to this use. Refer to Section 8.6.1 for more details.
Genetics sample collection (stored)	X	
Plasma phosphorylated tau (P-tau)	X	Samples may be stored and used to support the development of new diagnostic tools or assays. Stored samples may also be used to assess an individual's eligibility for future clinical trials if the individual consents to this use.

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1.3.2. Screening Period

Notes: Visit 1 should be conducted only for participants who are eligible for the study based on the result of the biomarker screening performed at Visit 601.

Screening Period		**Comment**
Visit number	**1**	Visit may occur over more than 1 day.
Timing	Completed within ≤56 days after availability of prescreening plasma P-tau result	
Visit interval tolerance (days)	—	
Informed consent of study partner	X	Relevant ICF must be signed before any protocol-specific tests or procedures are performed.
Register visit with IWRS	X	
Inclusion/exclusion criteria; confirmation of eligibility	X	Inclusion and exclusion criteria should be reviewed as information becomes available.
Preexisting conditions and medical history, including relevant surgical history	X	
Concomitant medications	X	
Adverse events (AEs)	X	Any events that occur after signing the ICF are considered AEs (Section 8.3). Additional data are collected for certain AEs (Section 8.3.3).
Mini-Mental State Examination (MMSE)	X	Must be administered in person at the study site by a member of the site staff trained to perform the MMSE.
Functional Activities Questionnaire (FAQ)	X	Must be administered in person at the study site by a member of the site staff trained to perform the FAQ.
The following activities are to be performed only for participants who meet both the MMSE and FAQ inclusion criteria		
Clinical Dementia Rating (CDR)	X	Remotely administered by central rater. Should be performed within 63 days before the first dose.
C-SSRS Baseline/Screening (Full Version)	X	Must be administered by a member of the site staff trained to perform the C-SSRS. C-SSRS should be completed after collection of unsolicited AEs (Section 8.2.7.1).
Imaging, laboratory tests, and sample collections		
MRI	X	Centrally read MRI is needed for study eligibility (Section 8.2.8).

Screening Period		**Comment**
Visit number	**1**	Visit may occur over more than 1 day.
Timing	Completed within ≤56 days after availability of prescreening plasma P-tau result	
Visit interval tolerance (days)	—	
ECG	X	Locally read (Section 8.2.2).
Hematology	X	
Clinical chemistry	X	
Urinalysis	X	
Stored samples		
Exploratory biomarkers	X	

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1.3.3. Double-Blind Treatment Period

Notes: Visit 2 should be conducted only for participants who are eligible for the study based on the results from screening at Visit 1.

Visits may occur over more than 1 day, provided that all procedures are completed within the visit interval tolerance. For participants who discontinue the study during this period, see ETa visit in Section 1.3.4 of the double-blind observation period. An ETa visit should be conducted if a participant has received at least 1 dose of study drug. If dosing is interrupted due to ARIA or any other reason, continue to perform visits and other study procedures per the SoA.

Visit 801 (follow-up) is described in Sections 1.3.5 and 4.1.5.

Double-Blind Treatment Period										**Comment**
Visit number	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	
Weeks relative to first dose	0	8	16	24	32	40	52	60	72	
Visit interval tolerance (days)	a	±7	±7	±7	±7	±7	±7	±7	±7	[a] V2 concludes no later than 63 days after the participant's V1 CDR assessment, with administration of the first dose of study intervention and collection of the postdose PK sample.
Inclusion/exclusion criteria; confirmation of eligibility	X									Inclusion and exclusion criteria should be confirmed during V2, prior to registering the visit and randomization via IWRS.
Register visit with IWRS	X	X	X	X	X	X	X	X	X	
Randomization via IWRS	X									
Concomitant medications	X	X	X	X	X	X	X	X	X	
Adverse events (AEs)	X	X	X	X	X	X	X	X	X	Any events that occur after signing the ICF are considered AEs (Section 8.3). Additional data are collected for certain AEs (Section 8.3.3).
Height	X									
Weight	X						X			
Vital signs	X	X	X	X	X		X			See Section 8.2.4.
Physical (including neurological) examination	X									Perform as described in Section 8.2.3.

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Double-Blind Treatment Period										**Comment**
Visit number	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	
Weeks relative to first dose	0	8	16	24	32	40	52	60	72	
Visit interval tolerance (days)	a	±7	±7	±7	±7	±7	±7	±7	±7	[a] V2 concludes no later than 63 days after the participant's V1 CDR assessment, with administration of the first dose of study intervention and collection of the postdose PK sample.
C-SSRS Since Last Assessed (Full Version)	X	X		X			X			Must be administered by a member of the site staff trained to perform the C-SSRS. C-SSRS should be completed after collection of unsolicited AEs but prior to dosing at on-site dosing visits (Section 8.2.7.1).
Clinical outcome assessments										V2 clinical assessments should be conducted at least 14 days after completion of the MMSE at V1. For V2, V3, V4, V5, and V6, on-site dosing should occur after all clinical outcome assessments for the visit have been completed. Clinical outcome assessments should be administered remotely by central raters or site staff. Certain outcome assessments may require a practice run prior to baseline.
Category Fluency	X			X			X			
Clinical Dementia Rating (CDR)				X			X			
ADCS-ADL-PI – Participant	X			X			X			
ADCS-ADL-PI – Study Partner	X			X			X			
Participant CFI	X						X			
Partner CFI	X						X			
QDRS – Participant	X			X			X			
QDRS – Study Partner	X			X			X			
Continuous Paired Associate Learning (CPAL)	X			X			X			
International Daily Symbol Substitution Test - Medicines (iDSSTm)	X			X			X			

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Double-Blind Treatment Period										**Comment**
Visit number	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	
Weeks relative to first dose	0	8	16	24	32	40	52	60	72	
Visit interval tolerance (days)	a	±7	±7	±7	±7	±7	±7	±7	±7	[a] V2 concludes no later than 63 days after the participant's V1 CDR assessment, with administration of the first dose of study intervention and collection of the postdose PK sample.
Cogstate International Shopping List Test (ISLT)	X			X			X			
Mild Behavioral Impairment Checklist (MBI-C)	X			X			X			
Montreal Cognitive Assessment (MoCA)	X			X			X			
Resource Utilization Inventory - Brief (RUI-B)	X			X			X			
Quality of Life in Alzheimer's Disease (QOL-AD) – Participant	X						X			
Quality of Life in Alzheimer's Disease (QOL-AD) – Study Partner	X						X			
Zarit Burden Interview (ZBI-22)	X						X			
Imaging, laboratory tests, and sample collections										

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Double-Blind Treatment Period										**Comment**
Visit number	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	
Weeks relative to first dose	0	8	16	24	32	40	52	60	72	
Visit interval tolerance (days)	a	±7	±7	±7	±7	±7	±7	±7	±7	[a] V2 concludes no later than 63 days after the participant's V1 CDR assessment, with administration of the first dose of study intervention and collection of the postdose PK sample.
MRI		X	X	X		X				Unscheduled MRIs may be performed at the discretion of the investigator. MRIs at V3, V4, V5, and V7 must be performed and reviewed within the visit window and within 7 days prior to dosing (Section 8.2.8). If dosing is interrupted and resumed • complete MRIs per the SoA • resume dosing as per the sequence described in this protocol, and • ensure MRIs (unscheduled MRIs), are conducted within 7 days prior to the second, third, fourth, fifth, and seventh doses. Refer to Section 7.1.2 for more details.
Hematology			X				X			
Clinical chemistry			X				X			
Urinalysis			X				X			
Immunogenicity (ADA)	X		X				X			If a systemic drug hypersensitivity reaction occurs, unscheduled samples should be collected (Section 8.3.3.2). Immunogenicity samples and PK samples for immunogenicity must be taken before dosing.

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Double-Blind Treatment Period										**Comment**
Visit number	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	
Weeks relative to first dose	0	8	16	24	32	40	52	60	72	
Visit interval tolerance (days)	a	±7	±7	±7	±7	±7	±7	±7	±7	[a] V2 concludes no later than 63 days after the participant's V1 CDR assessment, with administration of the first dose of study intervention and collection of the postdose PK sample.
PK sample – predose and postdose	X		X				X			If study drug is not administered on site at visits where PK samples are collected, collect a single sample using the "predose" sample tube and mark the postdose sample as not collected on the lab requisition form. Collect the postdose sample about 3 hours after dosing, or at the end of the study visit, whichever comes first. However, it is recommended to collect the postdose sample at least 30 minutes after dosing.
Stored samples										
Exploratory biomarker	X		X				X			
Study intervention compliance										
Dosing diary dispensed					X	X	X	X	X	Site personnel will provide training for the participant and/or study partner for SC dose administration (Section 6.1.1). Additional diaries can be dispensed as needed if dosing continues into the Double-Blind Observation Period due to a temporary discontinuation of study drug (Section 7.1.2).
Dosing diary returned and reviewed						X	X	X	X	The dosing diary is to be returned and reviewed within the visit window.
Dosing-related activities										
Dispense and administer study intervention on-site	X	X	X	X	X					At V7 and thereafter in this study period, dosing may be either on-site or off-site.

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Double-Blind Treatment Period										**Comment**
Visit number	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	
Weeks relative to first dose	0	8	16	24	32	40	52	60	72	
Visit interval tolerance (days)	a	±7	±7	±7	±7	±7	±7	±7	±7	[a] V2 concludes no later than 63 days after the participant's V1 CDR assessment, with administration of the first dose of study intervention and collection of the postdose PK sample.
Dispense study intervention to participant for Q4W dosing					X	X	X	X		See Section 7.1.2 about temporary discontinuation of study drug. If study drug is withheld, dispensing may occur at alternative timepoints as needed.
SC dose (mg)	400	400	800	800	800 Q4W is administered at Weeks 32, 36, 40, 44, 48, 52, 56, 60, 64, 68, and 72					Last planned dose is given at Week 72 in this period. Doses must be administered in the sequence specified in the protocol (Sections 4.1 and 6.1), even if study drug was temporarily withheld (Section 7.1.2). For V2 through V6, on-site dosing should occur after all other activities with the exception of postdose PK sample collection, if applicable.

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1.3.4. Double-Blind Observation Period

Notes: Up to 104 weeks of observation are planned for this study period. However, as needed, additional visits will be scheduled every 26 weeks after Visit 15 using the Extra Visit schedule in repeating sequence (EVa, then EVb) until the end of the study. The Early Termination visit for this period (ETa) should only be conducted if a participant has received at least 1 dose of study intervention. Visits may occur over more than 1 day, provided that all procedures are completed within the visit interval tolerance.

Visit 801 (follow-up) is described in Sections 1.3.5 and 4.1.5.

Double-Blind Observation Period									**Comment**
Visit number	**11**	**12**	**13**	**14**	**15**	**EVa**	**EVb**	**ETa**	
Weeks relative first dose	78	104	130	156	182	208	234	—	
Visit interval tolerance (days)	±28	±28	±28	±28	±28	±28	±28	—	
Register visit with IWRS	X	X	X	X	X	X	X	X	
Concomitant medications	X	X	X	X	X	X	X	X	
Adverse events (AEs)	X	X	X	X	X	X	X	X	Any events that occur after signing the ICF are considered AEs (Section 8.3). Additional data are collected for certain AEs (Section 8.3.3).
Weight		X		X		X		X	
Vital signs		X						X	See Section 8.2.4.
C-SSRS Since Last Assessed (Full Version)	X	X						X	Must be administered by a member of the site staff trained to perform the C-SSRS. C-SSRS should be completed after collection of unsolicited AEs (Section 8.2.7.1).
Clinical outcome assessments									Clinical outcome assessments should be administered remotely by central raters or site staff. At ETa, complete the clinical and cognitive assessments only if these were not already performed within the previous 90 days.
Category Fluency	X	X	X	X	X	X	X	X	
Clinical Dementia Rating (CDR)	X	X	X	X	X	X	X	X	
ADCS-ADL-PI – Participant	X	X	X	X	X	X	X	X	
ADCS-ADL-PI – Study Partner	X	X	X	X	X	X	X	X	

Double-Blind Observation Period									**Comment**
Visit number	**11**	**12**	**13**	**14**	**15**	**EVa**	**EVb**	**ETa**	
Weeks relative first dose	78	104	130	156	182	208	234	—	
Visit interval tolerance (days)	±28	±28	±28	±28	±28	±28	±28	—	
Participant CFI		X		X		X		X	
Partner CFI		X		X		X		X	
QDRS – Participant	X	X	X	X	X	X	X	X	
QDRS – Study Partner	X	X	X	X	X	X	X	X	
Continuous Paired Associate Learning (CPAL)	X	X	X	X	X	X	X	X	
International Daily Symbol Substitution Test - Medicines (iDSSTm)	X	X	X	X	X	X	X	X	
Cogstate International Shopping List Test (ISLT)	X	X	X	X	X	X	X	X	
Mild Behavioral Impairment Checklist (MBI-C)	X	X	X	X	X	X	X	X	
Montreal Cognitive Assessment (MoCA)	X	X	X	X	X	X	X	X	
Resource Utilization Inventory - Brief (RUI-B)		X		X		X		X	
Quality of Life in Alzheimer's Disease (QOL-AD) – Participant		X		X		X		X	
Quality of Life in Alzheimer's Disease (QOL-AD) – Study Partner		X		X		X		X	
Zarit Burden Interview (ZBI-22)		X		X		X		X	
Imaging, laboratory tests, and sample collections									
MRI		X						X	Unscheduled MRIs may be performed at the discretion of the investigator.
Hematology		X						X	
Clinical chemistry		X						X	
Urinalysis		X						X	

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Double-Blind Observation Period									**Comment**
Visit number	**11**	**12**	**13**	**14**	**15**	**EVa**	**EVb**	**ETa**	
Weeks relative first dose	78	104	130	156	182	208	234	—	
Visit interval tolerance (days)	±28	±28	±28	±28	±28	±28	±28	—	
Immunogenicity (ADA)		X						X	If a systemic drug hypersensitivity reaction occurs, unscheduled samples should be collected (Section 8.3.3.2). Immunogenicity samples and PK samples for immunogenicity must be taken before dosing.
PK sample – random		X						X	At ETa, collect PK sample if ETa occurs within 20 weeks of last dose.
Stored samples									
Exploratory biomarker		X							
Dosing-related activities									
No dosing in this study period									There is no planned dosing in this study period except as needed to complete the dosing regimen of the previous study period; see Section 7.1.2.

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1.3.5. Open-Label Extension Period and Visit 801

Open-label Extension Period

Notes: The optional Open-label Extension Period is available only to participants who meet certain criteria, including, but not limited to, having received placebo during the Double-Blind Treatment Period. See Section 4.1.4 for details. Informed consent for this study period must be obtained before performing any procedures specific to this study period. Visits may occur over more than 1 day, provided that all procedures are completed within the visit interval tolerance. If dosing is interrupted due to ARIA or any other reason, continue to perform visits and other study procedures per the SoA.

Visit 801

Note: Follow-up occurs at least 20 weeks from the last dose, as described in Section 4.1.5.

Open-Label Extension Period and V801													**Comment**
Visit number	**101**	**102**	**103**	**104**	**105**	**106**	**107**	**108**	**109**	**110**	**801**	**ETb**	
Weeks relative to first dose in this study period	0	8	16	24	32	40	52	60	72	78	—		
Visit interval tolerance (days)	±7	±7	±7	±7	±7	±7	±7	±7	±7	±28	—		
Informed consent participant – OLE	X												
Informed consent study partner – OLE	X												
Inclusion/exclusion criteria; confirmation of eligibility at V101	X												See "Open-label extension exclusion" criteria in Section 5.2. These criteria should be confirmed during V101, prior to registering the visit with IWRS.
Register visit with IWRS	X	X	X	X	X	X	X	X	X	X	X	X	
Concomitant medications	X	X	X	X	X	X	X	X	X	X	X	X	
Adverse events (AEs)	X	X	X	X	X	X	X	X	X	X	X	X	Any events that occur after signing the ICF are considered AEs (Section 8.3). Additional data are collected for certain AEs (Section 8.3.3).
Vital signs	X	X	X	X	X								See Section 8.2.4.

Open-Label Extension Period and V801													**Comment**
Visit number	**101**	**102**	**103**	**104**	**105**	**106**	**107**	**108**	**109**	**110**	**801**	**ETb**	
Weeks relative to first dose in this study period	0	8	16	24	32	40	52	60	72	78	—		
Visit interval tolerance (days)	±7	±7	±7	±7	±7	±7	±7	±7	±7	±28	—		
C-SSRS Since Last Assessed (Full Version)	X	X		X			X				X	X	Must be administered by a member of the site staff trained to perform the C-SSRS. C-SSRS should be completed after collection of unsolicited AEs but prior to dosing at on-site dosing visits (Section 8.2.7.1).
Clinical outcome assessments													Clinical outcome assessments must be administered remotely by central raters. At V101 and ETb, complete the clinical and cognitive assessments only if these were not already performed within the previous 90 days.
Clinical Dementia Rating (CDR)	X			X			X					X	
Montreal Cognitive Assessment (MoCA)	X			X			X					X	
Imaging, laboratory tests, and sample collections													
MRI	X	X	X	X		X					X	X	Unscheduled MRIs may be performed at the discretion of the investigator. At V101, MRI must be centrally read. MRIs at V101, V102, V103, V104, and V106 must be performed and reviewed within the visit window and within 7 days prior to dosing (Section 8.2.8). If dosing is interrupted and resumed

Open-Label Extension Period and V801													**Comment**
Visit number	**101**	**102**	**103**	**104**	**105**	**106**	**107**	**108**	**109**	**110**	**801**	**ETb**	
Weeks relative to first dose in this study period	0	8	16	24	32	40	52	60	72	78	—		
Visit interval tolerance (days)	±7	±7	±7	±7	±7	±7	±7	±7	±7	±28	—		
													• complete MRIs per the SoA • resume dosing as per the sequence described in this protocol, and • ensure MRIs (unscheduled MRIs), are conducted within 7 days prior to the second, third, fourth, fifth, and seventh doses. Refer to Section 7.1.2. for more details.
Hematology			X				X				X	X	
Clinical chemistry			X				X				X	X	
Urinalysis			X				X				X	X	
Immunogenicity (ADA)	X		X				X				X	X	If a systemic drug hyper-sensitivity reaction occurs, unscheduled samples should be collected (Section 8.3.3.2). Immunogenicity samples and PK samples for immunogenicity must be taken before dosing.
PK sample – predose and postdose	X		X				X						If study drug is not administered on site at visits where PK samples are collected, collect a single sample using the “predose” sample tube and mark the postdose sample as not collected on the lab requisition form.

Open-Label Extension Period and V801													**Comment**
Visit number	**101**	**102**	**103**	**104**	**105**	**106**	**107**	**108**	**109**	**110**	**801**	**ETb**	
Weeks relative to first dose in this study period	0	8	16	24	32	40	52	60	72	78	—		
Visit interval tolerance (days)	±7	±7	±7	±7	±7	±7	±7	±7	±7	±28	—		
													Collect the postdose sample about 3 hours after dosing, or at the end of the study visit, whichever comes first. However, it is recommended to collect the postdose sample at least 30 minutes after dosing.
PK sample – random											X	X	At ETb, collect PK sample if ETb occurs within 20 weeks of last dose.
Stored samples													
Exploratory biomarker	X		X				X						
Study intervention compliance													
Dosing diary dispensed					X	X	X	X					Site personnel will provide training for the participant and/or study partner for SC dose administration (Section 6.1.1).
Dosing diary returned and reviewed						X	X	X	X			X	The dosing diary is to be returned and reviewed within the visit window.
Dosing related activities													
Dispense and administer study intervention on-site	X	X	X	X	X								At V106 and thereafter in this study period, dosing may be either on-site or off-site.
Dispense study intervention to participant for Q4W dosing					X	X	X	X					See Section 7.1.2 about temporary discontinuation of study drug. If study drug is withheld, dispensing may occur at alternative timepoints as needed.

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Open-Label Extension Period and V801													**Comment**
Visit number	**101**	**102**	**103**	**104**	**105**	**106**	**107**	**108**	**109**	**110**	**801**	**ETb**	
Weeks relative to first dose in this study period	0	8	16	24	32	40	52	60	72	78	—		
Visit interval tolerance (days)	±7	±7	±7	±7	±7	±7	±7	±7	±7	±28	—		
SC dose (mg)	400	400	800	800	800 Q4W is administered at Weeks 32, 36, 40, 44, 48, 52, 56, 60, 64, 68, and 72								Last planned dose is given at Week 72 in this period. Doses must be administered in the sequence specified in the protocol (Sections 4.1 and 6.1), even if study drug was temporarily withheld (Section 7.1.2). For V101 through V105, on-site dosing should occur after all other activities with the exception of postdose PK sample collection, if applicable.

Abbreviations: ADA = anti-drug antibodies; ADCS-ADL-PI = ADCS Activities of Daily Living – Prevention Instrument; APOE = apolipoprotein E; ARIA = amyloid-related imaging abnormalities; CDR = Clinical Dementia Rating; CFI= Cognitive Function Index; C-SSRS = Columbia-Suicide Severity Rating Scale; ECG = electrocardiogram; ETa = early termination visit during Double-Blind Treatment Period or Double-Blind Observation Period; ETb = early termination visit during Open-Label Extension Period; EVa = Extra Visit, first, third, fifth, etc.; EVb = Extra Visit, second, fourth, sixth, etc.; ICF = informed consent form; IWRS = interactive web response system; MRI = magnetic resonance imaging; OLE = open-label extension; PK = pharmacokinetic; Q4W = every 4 weeks; QDRS = Quick Dementia Rating System; SC = subcutaneous; V = visit.

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2. Introduction

Remternetug is an IgG1 mAb directed at the pyroglutamate modification of the third amino acid of the amyloid-beta peptide (N3pG Aβ), which is present only in brain amyloid plaque, resulting in microglial-mediated clearance of brain amyloid plaque. Remternetug is being developed for the treatment of early AD.

2.1. Study Rationale

The pathophysiological process of AD begins more than a decade before the clinical stage now recognized as AD dementia (Bateman et al. 2011; Sperling et al. 2011; Villemagne et al. 2013). The initial period in which individuals have AD pathology prior to the onset of dementia is known as early AD. Consensus and emerging data in the field suggest that amyloid-targeting therapies may have greater benefit when started earlier in the disease continuum (Siemers et al. 2016; Irizarry et al. 2023; van Dyck et al. 2023).

Study J1G-MC-LAKI (LAKI) is a multicenter, randomized, double-blind, placebo-controlled, Phase 3 study to evaluate the safety and efficacy of remternetug in participants with early AD. The primary objective of Study LAKI will assess whether treatment with remternetug delays the time to clinically meaningful progression as compared to placebo.

2.2. Background

The mechanism of action of remternetug is to target and remove deposited amyloid plaque, a key pathological hallmark of AD, via microglial-mediated clearance. Donanemab, a mAb targeting a similar epitope as remternetug, reduced cerebral amyloid plaques and slowed cognitive and functional decline compared to placebo in 2 clinical trials of early symptomatic AD (Mintun et al. 2021; Sims et al. 2023) and has been approved by multiple countries, including the United States, Great Britain, and Japan, for the treatment of MCI and mild dementia due to AD. Lecanemab, a mAb-targeting Aβ, has also been approved by multiple countries for the treatment of MCI and mild dementia due to AD.

Remternetug has demonstrated robust amyloid plaque reduction (Jin Y et al. 2023) in participants with AD with a safety profile supporting further clinical development. See the IB for additional details regarding PK/PD, and safety from completed and ongoing studies of remternetug in healthy participants and participants with AD. The data available to date support the potential for remternetug as an amyloid-targeting therapy with a best-in-class benefit/risk profile. Study LAKC, particularly Addendum 5, is evaluating titration-based SC dosing in early symptomatic AD participants, and the PK/PD and safety data from that study supports remternetug for further evaluation in Study LAKI.

Recent data from studies of donanemab support that treatment earlier in the disease results in clinically meaningful slowing of disease progression as measured by the CDR-GS and the CDR-SB (Mintun 2023; Sims et al. 2023).

Individuals with early AD are a large population with significant unmet need. Thus, there is a need for a treatment that delays clinically meaningful cognitive and functional progression, prevents dementia, and allows individuals to remain independent longer. In addition, targeting an

early population with less amyloid burden is anticipated to result in a more favorable benefit-risk profile than targeting those with more advanced disease. Currently, there are no or limited disease-modifying therapies approved for use across early AD Stages 1 through 3, and a treatment that delays the onset of cognitive or functional impairment in early AD would have a large societal benefit (Zissimopoulos et al. 2014). Remternetug, with its potential for at-home administration via SC injection, represents an important treatment option for individuals with early AD. This study will evaluate the efficacy and safety of remternetug, an antibody that binds to and removes deposited amyloid plaque, in participants with early AD.

2.3. Benefit/Risk Assessment

Detailed information about the known and expected benefits and risks and reasonably expected AEs of remternetug may be found in the IB.

2.3.1. Risks and Mitigations

Study intervention

Amyloid-related imaging abnormalities (ARIA-E and ARIA-H) and macrohemorrhages

Monoclonal antibodies targeting amyloid plaques, including remternetug, have been associated with increased risk of ARIA and cerebral hemorrhages (also known as macrohemorrhages) in patients with AD.

MRI brain scans will be performed as routine monitoring for ARIA during this study. An unscheduled MRI should be obtained upon suspicion of ARIA. Participants will be informed of potential symptoms of ARIA and will be instructed to contact the investigative site if symptoms occur.

Investigators will be provided information about monitoring for, and managing events of, ARIA as well as about temporary or permanent discontinuation of study intervention for ARIA. Participants who develop macrohemorrhage will be permanently discontinued from study intervention (Section 7.1).

Hypersensitivity, immediate and nonimmediate, including anaphylaxis

Hypersensitivity events may occur with large molecules. Though remternetug has not been associated with significant hypersensitivity reactions, investigators will monitor participants for hypersensitivity reactions. If a hypersensitivity reaction occurs, appropriate medication per local standard of care may be used. The protocol provides instructions regarding premedication for administration of study intervention (Section 6.1.1).

Injection site reactions

Mild or moderate injection site reactions have been observed after SC administration of remternetug. Symptomatic management may be considered if indicated.

Additional risk mitigation

To date, no clinically meaningful immunogenicity has been observed with remternetug; however, immunogenicity will be monitored in this study.

An external DMC will oversee the conduct of this study (Appendix 1, Section 10.1.5).

Study procedure risks

Magnetic resonance imaging (MRI)

MRI scans will be performed as routine safety monitoring during the study. AEs with MRI procedures are rare. However, the procedure involves a strong, magnetic pulsed gradient field which presents risks for participants with metal implants or other accessory medical devices, such as pacemakers, stents, and cochlear implants. Participants with contraindications for MRI are excluded from this study (Section 5.2).

The confined space of the MRI testing environment may cause some participants to experience claustrophobia, and some participants may find remaining still to be difficult for the duration of the imaging process.

2.3.2. Benefit Assessment

The efficacy of remternetug in AD has not yet been established. Time-dependent and dose-dependent amyloid plaque lowering has been observed with remternetug treatment; see the IB for details.

Participants may benefit by receiving personal health information from the routine safety assessments performed in this study.

2.3.3. Overall Benefit Risk Conclusion

Measures are taken to minimize risk to individuals participating in this study. The potential risks identified in association with remternetug treatment are justified by the anticipated benefits that may be afforded to the early AD population by removing amyloid plaque.

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3. Objectives, Endpoints, and Estimands

Objectives	Endpoints
Primary	
To assess the effect of remternetug versus placebo on time to clinically meaningful progression in participants with early AD	Time to clinically meaningful progression as measured by CDR
Secondary	
To assess the effect of remternetug versus placebo on other measures of clinical progression in participants with early AD	Change from baseline through the participant's final visit, for these clinical outcome assessments: ● CDR-SB ● MoCA ● Category Fluency ● CPAL ● ISLT ● iDSSTm ● CFI ● MBI-C ● ADCS-ADL-PI ● Cognitive composite score involving a combination of the individual assessments above may be analyzed. Additional details will be provided in the SAP.
To describe the safety of remternetug	Standard safety assessments, including ● Spontaneously reported AEs ● clinical laboratory tests ● vital signs MRI (ARIA and emergent radiological findings) Hypersensitivity reactions Injection site reactions C-SSRS
To assess peripheral PK and presence of anti-remternetug antibodies	PK of remternetug TE ADA against remternetug

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Abbreviations: AD = Alzheimer's Disease; ADA = anti-drug antibodies; ADCS-ADL-PI = ADCS Activities of Daily Living – Prevention Instrument; AEs = adverse events; ARIA = amyloid-related imaging abnormalities; CDR = Clinical Dementia Rating; CDR-SB = CDR-sum of boxes; CFI = Cognitive Function Index; CPAL = Continuous Paired Associate Learning; C-SSRS = Columbia-Suicide Severity Rating Scale; iDSSTm = International Daily Symbol Substitution Test-Medicines; ISLT = International Shopping List Test; MBI-C = Mild Behavioral Impairment Checklist; MoCA = Montreal Cognitive Assessment; MRI = magnetic resonance imaging; PK = pharmacokinetic; SAP = statistical analysis plan; TE = treatment-emergent.

Primary estimand

The primary clinical question of interest is: *What is the difference in time to clinically meaningful progression between participants with early AD who receive remternetug versus those who receive placebo, regardless of change in permitted concomitant medications and discontinuation of the study intervention other than discontinuation due to the use of a prohibited medication?*

The estimand is described by the following attributes:

- **Population:** Individuals with early AD.
- **Endpoint:** Time to clinically meaningful progression as measured by an increase from baseline at two consecutive time points either in CDR-SB of 1 or greater or in CDR-GS of 0.5 or greater.
- **Treatment condition:** The randomized treatment with or without change in permitted concomitant medications and without use of prohibited medication.
- **Intercurrent events:** The following intercurrent events are considered and are addressed by the strategies specified.
  - Use of any anti-amyloid immunotherapies (outlined in Section 6.9.2, Prohibited Medications), other than remternetug, prior to attaining clinically meaningful progression is addressed by the while-on-treatment strategy.
  - The change in permitted concomitant medications (outlined in Section 6.9.1, Permitted Concomitant Therapies) and discontinuation of study intervention for reasons other than due to the use of a prohibited medication are both addressed by the treatment policy strategy.
  - An intercurrent event of participant death will be handled using a composite strategy.

    No other intercurrent events are considered.
- **Population-level summary:** The hazard ratio between remternetug and placebo on time to clinically meaningful progression.

Primary endpoint analyses are described in Section 9.3.2. Additional details regarding the estimand are specified in the SAP.

A sensitivity analysis using an estimand strategy for use of prohibited medications as an intercurrent event, other than while-on-treatment strategy, will be conducted.

Secondary estimands

Secondary estimands, if any, will be described in the SAP.

4. Study Design

4.1. Overall Design

This is a randomized, parallel-group, double-blind, 2-arm, placebo-controlled, Phase 3 study of remternetug in participants who have early AD (AD Stages 1 through 3).

The study has the following periods:

- Prescreening/Screening — This period includes Visit 601 and Visit 1.
- Double-Blind Treatment — This period starts with Visit 2 and continues for approximately 78 weeks.
- Double-Blind Observation — This period starts at the end of Double-Blind Treatment Period and continues until the target number of events for the primary endpoint is achieved or until the study is ended at the sponsor's discretion.
- Open-Label Extension (only for participants who previously received placebo) — This period will be conducted at the sponsor's discretion, contingent on the availability of confirmatory efficacy results. If started, this period may continue for approximately 78 weeks.
- Follow-up (Visit 801) — This period occurs at least 20 weeks after a participant's last dose of study intervention. If, at the time of study completion or early discontinuation more than 20 weeks have passed since the participant's last dose of study intervention, Visit 801 is not required.

Participants who meet study entry criteria will be randomized in a 1:1 ratio to one of these double-blind treatment groups:

- Remternetug, given at a total of up to 15 time points in the following sequence:
  - first dose - 400 mg at randomization (Week 0)
  - second dose - 400 mg at 8 weeks after randomization (Week 8)
  - third dose - 800 mg at 16 weeks after randomization (Week 16)
  - fourth dose - 800 mg at 24 weeks after randomization (Week 24)
  - fifth dose - 800 mg at 32 weeks after randomization (Week 32)
  - sixth dose - 800 mg at 36 weeks after randomization (Week 36)
  - seventh dose - 800 mg at 40 weeks after randomization (Week 40), and
  - eighth through fifteenth doses - 800 mg at every fourth week thereafter, up to and including 72 weeks after randomization (Weeks 44, 48, 52, 56, 60, 64, 68, and 72).
- Placebo, given at matching time points (during the double-blind treatment period).

If a dose is withheld for any reason, participants should receive doses in the sequence outlined above. See Section 7.1.2 for additional information on temporary discontinuation.

Participants will be followed until the target number of primary outcome events is observed (Section 9.5). Therefore, the total duration of study participation for each participant will vary. Participants assigned to remternetug at randomization who have not completed dosing will be able to continue dosing until complete.

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In the Open-Label Extension Period, eligible participants who previously received placebo will receive remternetug at a total of up to 15 time points, as described in Section 4.1.4.

This study design includes blinding of investigators/assessors, study partners, and participants.

4.1.1. Prescreening and Screening Periods

The prescreening and screening periods consist of Visit 601 and Visit 1. The study will be explained to the participant and study partner. Written informed consent must be obtained before any study procedures are conducted. Procedures to be performed at these visits are indicated in the SoA.

Visit 601 includes a biomarker sample collection. Visit 1 can commence only after the participant's biomarker eligibility is confirmed.

4.1.2. Double-Blind Treatment Period

The Double-Blind Treatment Period begins at Visit 2 (randomization/baseline). At Visit 2, all remaining study visits after randomization should be scheduled as close as possible to the target date relative to first injection. Participants who meet entry criteria will be randomized to receive either remternetug or placebo at a total of up to 15 time points during this period.

4.1.3. Double-Blind Observation Period

The Double-Blind Observation Period begins at Visit 11 (Week 78). In this period, participants will receive neither remternetug nor placebo, unless a dose interruption occurs, and dosing is extended into the double-blind observation period (Section 7.1.2). Participants will have routine visits approximately every 26 weeks for clinical and safety assessments during this period.

Participants will continue in this period until the minimum number of participants are enrolled in the study and the target number of events for the primary endpoint is achieved, up to approximately 234 weeks since the first dose, at the sponsor's discretion.

Investigators will be notified when participants have reached their final observation visit or the sponsor ends the study.

4.1.4. Open-Label Extension Period

Activation of the Open-Label Extension Period is dependent on confirmatory results of remternetug efficacy at sponsor's discretion.

If this period is activated, participants who meet all of the following criteria may opt to receive remternetug:

- Participant received placebo in the Double-Blind Treatment Period.
- Participant is active in the Double-Blind Observation Period when the target number of primary outcome events is observed.
- Participant provides informed consent for the Open-Label Extension Period.
- Participant is not excluded from the Open-Label Extension Period, according to the relevant criteria specified for the Open-Label Extension Period in Section 5.2.

For eligible participants, remternetug will be given at a total of up to 15 time points:

Approved on 29 Aug 2025 GMT

- 400 mg at Visit 101 (Week 0 in this period)
- 400 mg at Visit 102 (Week 8 in this period)
- 800 mg at Visit 103 (Week 16 in this period)
- 800 mg at Visit 104 (Week 24 in this period)
- 800 mg at Visit 105 (Week 32 in this period), and
- 800 mg at every fourth week thereafter, up to and including 72 weeks after Week 0 of this period (Weeks 36, 40, 44, 48, 52, 56, 60, 64, 68, 72 in this period).

4.1.5. Visit 801

Participants who have received at least 1 dose of study intervention will have a follow-up visit (Visit 801). This visit is to occur at least 20 weeks from the last dose of study intervention.

If, however, at the time of study completion or early discontinuation more than 20 weeks have passed since the participant's last dose of study intervention, Visit 801 is not required.

4.2. Scientific Rationale for Study Design

Study LAKI is intended to evaluate the benefits and risks of treatment with remternetug in a population with early AD.

Amyloid plaque is a known hallmark pathology of AD, and is hypothesized to contribute to the cognitive and functional decline in people with AD. Therefore, it is hypothesized that the removal of amyloid by remternetug may reduce the risk of clinically meaningful progression.

Study population

Study LAKI will enroll participants with early AD, encompassing preclinical AD and MCI due to AD. This definition corresponds to AD Stages 1 through 3 from recent guidelines of the Alzheimer's Association Working Group (Jack et al. 2024) and is consistent with the International Working Group recommendations (Dubois et al. 2021) as well as with FDA guidance on AD drug development (FDA 2024). AD Stages 1 through 3 encompass individuals with evidence of AD pathology based on a set of validated core AD biomarkers and prior to the onset of dementia. These earliest stages of AD reflects a continuum, from normal cognition to early cognitive symptoms, with data suggesting 18% to 30% of individuals diagnosed with MCI will revert to normal cognition. This tendency for individuals to fluctuate between clinical stages makes clear differentiation between Stages 1 through 3 challenging in the clinical setting (Malek-Ahmadi 2016; Wood 2016; Yu et al. 2024). Therefore, treatment of early AD represents a straightforward treatment paradigm where patient selection would require simply evidence of AD pathology and the absence of dementia. Plasma P-tau is an accepted method and core AD biomarker for assessing AD pathology (Jack et al. 2024; Palmqvist et al. 2020) and will be used in this study for the purpose of participant eligibility (Section 5.1).

Primary outcome measure

The primary outcome measure is clinically meaningful progression, defined as an increase from baseline at two consecutive time points either in CDR-SB of 1 or greater or in CDR-GS of 0.5 or greater. CDR is an instrument that meaningfully assesses both daily function and cognition across all stages of the AD continuum. Both an increase in CDR-GS (reflecting a change in disease stage) and an increase in CDR-SB of ≥1 are considered clinically meaningful in early

AD (Andrews et al. 2019). These endpoints were chosen based on their intrinsic clinical meaningfulness and anticipated event rates in early AD.

4.2.1. Justification of Use of Placebo

Inclusion of a placebo treatment arm is acceptable because there are currently no or limited disease-modifying treatments approved for use across early AD Stages 1 through 3. This approach is consistent with the use of placebo described in the Declaration of Helsinki (WMA 2022). The use of a placebo comparator in this study is necessary to determine the efficacy and safety of remternetug therapy. Participants randomized to placebo may receive remternetug in an extension period (Section 4.1.4).

4.2.2. Rationale for Collection of Race and Ethnicity

In this study, the collection of demographic information includes ethnicity (where permissible according to local regulations) and race. The scientific rationale is based on the need to assess variable response in safety, efficacy, or both, among different races and ethnicities. Such a need can be addressed only if all the relevant data are collected.

4.3. Justification for Dose

The dose regimen planned for this protocol was selected in consideration of the following factors:

- Safety and tolerability data from Phase 1 and 3 clinical studies in participants with early symptomatic AD, and
- PK/PD (amyloid plaque) analyses of all available data across a wide dose range.

The proposed dosing regimen titration, initiating at low dose and increasing to higher doses and increasing frequency over time, has been demonstrated to mitigate ARIA risk with remternetug. The development of ARIA across amyloid targeted therapies is generally observed at the beginning of treatment (Salloway et al. 2022), and this has been observed with remternetug also. It is hypothesized that 8-week dosing intervals early in treatment will allow time for potential asymptomatic ARIA to resolve and may decrease the risk of symptomatic or serious ARIA, while more frequent dosing later in the dosing regimen has been observed to be associated with a low risk of ARIA with remternetug.

The SC titration treatment regimen is anticipated to result in approximately 86% of participants reaching amyloid plaque clearance at Week 76.

See the IB for additional details of the PK/PD and safety of remternetug that informs the proposed dosing regimen.

4.4. End of Study Definition

The end of the study, that is, end of treatment, is defined as the date of the last visit of the last participant in the study, including the Open-Label Extension Period (Section 4.1.4) and Follow Up Period (Section 4.1.5), where applicable.

End of study – primary outcome

The end of the study for the primary outcome occurs when the target number of primary outcome events is observed.

5. Study Population

Screening evaluations

Participant eligibility for enrollment in the study is based on the criteria listed in this section. The inclusion and exclusion criteria used to determine eligibility should only apply at screening or other specified visits, and not continuously throughout the study.

Prospective approval of protocol deviations to recruitment and enrollment criteria, also known as protocol waivers or exemptions, is not permitted.

All screening evaluations must be completed and reviewed to confirm that potential participants meet all eligibility criteria.

The investigator will maintain a screening log to record details of all participants screened and to confirm eligibility or record reasons for screening failure, as applicable.

Screening restrictions for certain demographics

Screening of individuals from 55 to 64 years of age, inclusive, may be restricted at the discretion of the sponsor.

Screening extensions for certain demographics

Screening of individuals from certain geographies may be extended at the discretion of the sponsor.

Screening of individuals from under-represented groups may be extended to optimize the diversity of the study population, also at the discretion of the sponsor.

Enrollment limits based on CDR scores

Enrollment of individuals with CDR-GS >0 may be restricted at the sponsor's discretion up to approximately one-third of the enrolled participants.

5.1. Inclusion Criteria

Individuals are eligible to be included in the study only if all of the following criteria apply:

Informed consent

[1] Are capable of giving, and have given, signed informed consent as described in Appendix 1, Section 10.1.3, which includes compliance with the requirements and restrictions listed in the ICF and in this protocol.

Note: Contraceptive use by participants should be consistent with local regulations regarding the methods of contraception for those participating in clinical studies. For the contraception requirements of this protocol, see Appendix 4, Section 10.4.2.

Participant characteristics

[2] Are from 55 to 80 years of age, inclusive, at the time of signing the ICF.

[3] Have venous access sufficient to allow the protocol-required blood sampling.

[4] Have adequate literacy, vision, and hearing for neuropsychological testing in the opinion of the investigator at the time of screening.

[5] Are reliable, willing, and able to make themselves available for the duration of the study and are willing to follow study procedures, including use of electronic devices to complete study-related questionnaires and video calls.

[6] Have at least 1 reliable study partner who meets all of these criteria:

- attained the legal age for consent to study participation
- has given their own written informed consent and is capable of complying with the requirements listed in the ICF and in this protocol
- is in frequent contact with the study participant and familiar with the participant's overall function and behavior, such as day-to-day activities and cognitive abilities
- is available to provide oversight for at-home SC injections and is willing and able to administer the injections, if needed and
- is available to complete the study assessments required of the study partner.

A second consenting study partner may serve as backup. If a study partner must withdraw from study participation, a qualifying replacement should be identified and approved at the investigator's discretion. The replacement will need to give their own written informed consent before initiating any study activities.

Disease-specific characteristics

[7] Have, at Visit 601, a plasma P-tau result consistent with the presence of abnormal brain amyloid pathology.

[8] Have, at Visit 1, an MMSE score ≥27.

[9] Have, at Visit 1, an FAQ score <6.

[10] If currently receiving medications as symptomatic treatment for AD (for example, cholinesterase inhibitors), dose has been stable for at least 30 days before Visit 1 and dose is planned to remain stable for the duration of the study.

5.2. Exclusion Criteria

Individuals are excluded from the study if any 1 of the following criteria apply:

Medical conditions

[11] Have dementia or significant other neurological disease that can affect cognition.

[12] Have current serious or unstable illnesses, including cardiovascular, hepatic, renal, gastroenterologic, respiratory, endocrinologic, neurologic (other than AD), psychiatric, immunologic, or hematologic disease and other conditions that, in the investigator's opinion, could interfere with the analyses in this study; or has a life expectancy of approximately ≤5 years.

[13] Have a history of cancer that, in the investigator's opinion, has a high risk of recurrence and preventing completion of the study.

[14] Have any of the following:

- are, in the judgment of the investigator, actively suicidal and therefore deemed to be at significant risk for suicide.
- have answered "yes" to either Question 4 or Question 5 on the "Suicidal Ideation" portion of the C-SSRS **and** the ideation occurred within the past month, or
- have answered "yes" to any of the suicide-related behaviors on the "suicidal behavior" portion of the C-SSRS **and** the behavior occurred within the past month.

[15] Have a history of clinically significant multiple or severe drug allergies, significant atopy, or severe posttreatment hypersensitivity reactions (including, but not limited to, erythema multiforme major, linear IgA dermatosis, toxic epidermal necrolysis, and/or exfoliative dermatitis).

Laboratory tests, vital signs, physical examinations, and imaging

[16] Have any clinically important abnormality as determined by investigator, prior to randomization, in vital signs, ECG, clinical laboratory test results, physical examination, or MRI that could be detrimental to the participant or could compromise the study.

[17] Have 1 or more of these laboratory test results at screening:

- ALT ≥2.5 times ULN
- AST ≥2.5 times ULN
- TBL ≥1.5 times ULN
- ALP ≥2 times ULN

Note: Participants with TBL ≥1.5 times ULN are not excluded if they meet **all** of the following criteria for Gilbert syndrome:

- bilirubin is predominantly indirect (unconjugated) at screening (direct bilirubin within normal limits)
- liver disease is absent
- ALT, AST, and ALP are all ≤1 times ULN at screening, and
- hemoglobin is not significantly decreased at screening.

[18] Have any contraindications for MRI, including claustrophobia or the presence of contraindicated metal (ferromagnetic) implants/cardiac pacemaker incompatible with MRI or without access to an MRI facility that accepts patients with pacemakers.

[19] Have a centrally read MRI demonstrating presence of ARIA-E, >1 cerebral microhemorrhages, any superficial siderosis, any macrohemorrhage, or severe white matter disease at screening.

Prior or current therapies

[20] Have ever had prior treatment with a passive anti-amyloid immunotherapy.

[21] Have received active immunization against Aβ in any other study.

[22] Have known allergies to remternetug, related compounds, or any components of the formulation, as described in the IB.

Previous or concurrent clinical trial participation

[23] Are currently enrolled in any other medical research (interventional or observational) judged not to be scientifically or medically compatible with this study.

[24] Have participated, within the last 30 days prior to Visit 1, in a clinical trial involving a study intervention judged not to be scientifically or medically compatible with Study LAKI. If the previous study intervention is scientifically or medically incompatible with Study LAKI, then at least 3 months or 5 half-lives (whichever is longer) should have passed before the participant is screened in Study LAKI. Recent participation in an observational study may be permitted upon review of the observational study protocol and upon approval by the sponsor.

[25] Have previously been randomly assigned to treatment and completed or withdrawn from this study or received remternetug in this or any prior investigational study.

Other exclusions

[26] Are of childbearing potential, as defined in Appendix 4, Section 10.4.1.

[27] Are pregnant or breastfeeding, or plan to become pregnant or to breastfeed during the study.

[28] Are site personnel directly involved with this study or are members of their immediate family, or have study partners who are site personnel directly involved in this study. Immediate family is defined as a spouse, parent, child, or sibling, whether biological or legally adopted.

[29] Are Lilly employees directly involved in this study, or have study partners who are Lilly employees directly involved in this study.

5.2.1. Criteria for Open-Label Extension Period

Open-label extension inclusion criteria

Individuals are eligible to be included in the open-label extension period of the study only if they meet all entry criteria for the double-blind treatment period prior to enrollment and if all of the following criteria apply:

[1], [5], and [6] as stated above in Section 5.1.

Open-label extension exclusion criteria

Individuals are excluded from participation in the open-label extension period if any 1 of the following criteria applies:

[18], [22], or [25], as stated above in Section 5.2, or

[30] or [31] as stated below.

[30] Have a centrally read MRI demonstrating presence of ARIA-E, >1 cerebral microhemorrhages, any superficial siderosis, any macrohemorrhage, or severe white matter disease at Visit 101.

[31] Are study participants who develop an illness before the start of Open-Label Extension Period that is unstable or serious or that, in the judgment of the investigator, precludes treatment with remternetug.

5.3. Lifestyle Considerations

Participants should refrain from donating blood or blood products from the time of screening until 20 weeks following the last dose of study intervention.

5.4. Screen Failures

A screen failure occurs when a participant who consents to participate in the clinical study is not subsequently assigned to a study intervention. A minimal set of screen failure information is required to ensure transparent reporting of screen failure participants to meet the Consolidated Standards of Reporting Trials publishing requirements and to respond to queries from regulatory authorities. Minimal information includes demography, screen failure details, and any SAEs.

Rescreening

Rescreening is permitted if an exclusion criterion has resolved or is no longer relevant, after consultation with the sponsor.

At each instance of rescreening, the individual being rescreened and study partner will be required to sign a new ICF (Appendix 1, Section 10.1.3) and will be assigned a new participant number.

5.5. Criteria for Temporarily Delaying Randomization of a Participant

Not applicable.

6. Study Interventions and Concomitant Therapy

Study intervention is defined as any medicinal products or medical devices intended to be administered to or used by a study participant according to the study protocol.

6.1. Study Interventions Administered

This table lists the interventions used in this clinical study.

Intervention name	Remternetug	Placebo to match
Dosage levels	400 mg and 800 mg	not applicable
Dose frequency	400 mg at Week 0 (randomization) and Week 8, followed by 800 mg at Week 16, Week 24, Week 32, and every fourth week thereafter, up to and including Week 72	volume and frequency as needed to maintain blind
Route of administration	Subcutaneous	subcutaneous
Authorized as defined by European Union Clinical Trial Regulation	not authorized in European Union	not authorized in European Union

Remternetug is given at a total of up to 15 time points in the following sequence in Study LAKI:

- first dose - 400 mg at randomization (Week 0)
- second dose - 400 mg at 8 weeks after randomization (Week 8)
- third dose - 800 mg at 16 weeks after randomization (Week 16)
- fourth dose - 800 mg at 24 weeks after randomization (Week 24)
- fifth dose - 800 mg at 32 weeks after randomization (Week 32)
- sixth dose - 800 mg at 36 weeks after randomization (Week 36)
- seventh dose - 800 mg at 40 weeks after randomization (Week 40), and
- eighth through fifteenth doses - 800 mg at every fourth week thereafter, up to and including 72 weeks after randomization (Weeks 44, 48, 52, 56, 60, 64, 68, and 72).

Placebo, given at matching time points (during the double-blind treatment period).

If a dose is withheld for any reason, participants should receive doses in the sequence outlined above. See Section 7.1.2 for additional information on temporary discontinuation.

Dosing intervals

Dosing intervals for SC Q8W dosing:

- SC Q8W dosing should occur on approximately the same day every other month (for example, every 56 days).
- The minimum interval between SC Q8W dosing is 51 days from the prior dose. Dosing must not occur at an interval of less than 51 days at any time during the Q8W dosing period.

Dosing intervals for SC Q4W dosing:

- SC Q4W dosing should occur on approximately the same day each month (for example, every 28 days).
- The minimum interval between SC Q4W dosing is 24 days from the prior dose. Dosing must not occur at an interval of less than 24 days at any time during the Q4W dosing period.

If maintaining the minimum interval between doses results in a visit being conducted out of a window of ≤7 days, it will not be considered a protocol deviation.

Packaging and labeling

Interventions will be provided in prefilled syringes. Study interventions (remternetug and placebo) will be

- supplied in accordance with current Good Manufacturing Practice, and
- labeled as appropriate for country requirements.

6.1.1. Administration Details

Premedication before dosing

Premedication for administration of study intervention is not planned.

However, if a participant has a hypersensitivity reaction, appropriate premedication per the local standard of care may be used, as determined by the study investigator, for that participant's subsequent injections.

Any premedication or medication used to treat a hypersensitivity reaction will be documented as a concomitant therapy (Section 6.9).

Location for administration of study intervention

This table describes who may administer study intervention to the participant and where.

Instance of dosing	Dose Level	Location	Administrator
First	400 mg	on-site	• study site personnel, or • participant (self-administration) or study partner under the supervision of study site personnel
Second	400 mg		
Third	800 mg		
Fourth	800 mg		
Fifth	800 mg		
Sixth and subsequent	800 mg	off-site or on-site	The investigator must assess the ability of the participant to self-administer injections. At the discretion of the investigator, injections may be administered by • participant (independent self-administration), or • participant (self-administration under supervision of study partner), or • study partner At the request of the participant/study partner, injections may be administered by study site personnel (on-site).

Training of participants and study partners for off-site dosing

Detailed instructions for participants and their study partner on administration of SC injections will be provided by the sponsor. The study site personnel will train participants and their study partner on SC administration of study intervention prior to the first off-site dose. The investigator must assess the ability of the participant to self-administer the injections; if needed, the study partner must be available to provide oversight for administration and be willing and able to administer the injections if the participant cannot.

6.2. Preparation, Handling, Storage, and Accountability

The investigator or designee must confirm appropriate storage conditions have been maintained during transit for all study intervention received and any discrepancies are reported and resolved before use of the study intervention.

Only participants enrolled in the study may receive study intervention. Only authorized study personnel may supply, prepare, or administer study intervention.

Study intervention stored and/or administered on site must be stored in a secure, environmentally controlled, and monitored (manual or automated) area in accordance with the labeled storage conditions with access limited to the investigator and authorized study personnel. Participants and study partners will receive instructions for storing intervention at home, as applicable.

The investigator or authorized study personnel are responsible for study intervention accountability, reconciliation, and record maintenance, that is, receipt, reconciliation, and final disposition records.

Further guidance and information for the final disposition of unused study will be provided by the sponsor.

6.3. Assignment to Study Intervention

Randomization

Participants will be given a participant number at Visit 601 and those who meet criteria for enrollment will be randomly assigned to study intervention (remternetug or placebo in a 1:1 ratio) at Visit 2. Assignment to treatment groups will be determined by a computer-generated random sequence using an IWRS. Site personnel will confirm that they have located the correct packages by entering a confirmation number found on the label into the IWRS.

Stratification

Participant randomization will be stratified by

- CDR-GS = 0 versus >0
- *APOE* ε4 status (homozygous, heterozygous, and noncarriers), and
- Geographic region (up to 4 regions, as defined by the sponsor).

6.4. Blinding

This is a double-blind study. Study intervention will be provided to sites and will be administered by blinded qualified site staff, participant, or participant's study partner.

To preserve the blinding of the study, a minimal number of Lilly personnel will see the randomization table and treatment assignments before the study is complete. The independent external DMC is unblinded to participants' randomly assigned treatments.

Emergency unblinding

Emergency unblinding at a participant level for AEs may be performed through the IWRS. This option may be used ONLY if management of the participant's safety would be influenced by knowledge of treatment assignment. All unblinding events are recorded and reported by the IWRS.

In case of an emergency, the investigator has the sole responsibility for determining if unblinding of a participant's treatment assignment is warranted for medical management of the event. The participant's safety must always be the first consideration in making such a determination. If a participant's treatment assignment is unblinded, Lilly must be notified immediately. If the investigator decides that unblinding is warranted, it is the responsibility of the investigator to promptly document the decision and rationale and notify Lilly as soon as possible.

Discontinuation of study intervention in instances of unblinding

If an investigator, site personnel performing assessments, or participant is unblinded, the investigator must obtain specific approval from a designated medical monitor for the participant to continue in the study.

Blinding/unblinding plan

Additional details regarding unblinding procedures will be specified in a separate blinding and unblinding plan.

6.5. Study Intervention Compliance

The first 5 doses of study intervention will be administered by or under supervision of the study site personnel. The assigned study intervention packages and study participant identification number will be confirmed prior to dosing and then dosing details will be entered into the electronic data capturing system.

For subsequent doses, study intervention may be dispensed to the participant or study partner for administration. The assigned study intervention packages and study participant identification number will be confirmed prior to dispensing. Each participant or study partner will receive a dosing diary to record dose administration details. Site staff should remind participants and study partners to bring the diary and any unused prefilled syringes to on-site visits for diary returns and reviews at the time points specified in the SoA. Site staff will assess dosing compliance during the dosing diary reviews and enter the dosing diary data from paper source document into electronic data capturing system.

6.6. Dose Modification

Dose modification is not permitted.

6.7. Continued Access to Study Intervention after the End of the Study

There will be no continued access to study intervention after the end of the study.

6.8. Treatment of Overdose

For this study, any dose of study intervention greater than the dose assigned will be considered an overdose. Lilly does not recommend specific treatment for a remternetug overdose. There is no known antidote to remternetug overdose.

In the event of an overdose, the investigator or treating physician should:

- contact the medical monitor immediately
- evaluate the participant to determine, in consultation with the sponsor's designated medical monitor, whether study intervention should be interrupted or whether the dose should be reduced, and
- closely monitor the participant for any AE or SAE and laboratory abnormalities as medically appropriate for approximately 20 weeks.

6.9. Prior and Concomitant Therapy

Any medication or vaccine, including over-the-counter or prescription medicines, vitamins, or herbal supplements that the participant is receiving at the time of enrollment or receives during the study must be recorded in the CRF.

The sponsor's designated medical monitor should be contacted if there are any questions regarding concomitant or prior medications.

Participants and their study partners will be instructed to consult the investigator or other appropriate study personnel at the site before initiation of any new medications or supplements and before changing dose of any current concomitant medications or supplements.

6.9.1. Permitted Concomitant Therapies

Medications as symptomatic treatment for AD

Participants may receive prescription medications as symptomatic treatment for AD (for example, cholinesterase inhibitors) during the study.

Participants who, at the time of randomization, were receiving a stable dose of prescription medication as symptomatic treatment for AD are encouraged to remain on the same stable dose throughout the study, unless a dose change is needed in the opinion of the investigator.

After randomization, participants who require new or increased doses of prescription medications as symptomatic treatment for AD may receive them and continue in the study on study intervention, if applicable.

Nonprescription supplements are permitted.

Nonmedication therapies for AD

Nonmedication therapies for AD, such as behavioral management, are permitted.

Treatment for symptomatic ARIA-E

Symptomatic ARIA-E may be treated with oral or intravenous steroids at the discretion of the investigator or treating physician per the Manual of Operations. Any medication used to treat symptomatic ARIA-E will be documented as a concomitant therapy.

Premedication or any medication to treat a hypersensitivity reaction

Any premedication or medication used to treat a hypersensitivity reaction (Section 6.1.1) will be documented as a concomitant therapy.

6.9.2. Prohibited Medications

Concurrent use of anti-amyloid immunotherapies, other than remternetug, is not permitted during this study. Such therapies include but are not limited to

- gantenerumab
- lecanemab
- aducanumab, or
- donanemab.

7. Discontinuation of Study Intervention and Participant Discontinuation/Withdrawal

Discontinuation of specific sites or of the study as a whole are handled as part of Appendix 1, Section 10.1.9.

7.1. Discontinuation of Study Intervention

When necessary, a participant may be temporarily or permanently discontinued from the study intervention based on the considerations in the following subsections.

If a participant permanently discontinues study intervention (treatment), the participant should be encouraged to remain in the study and follow procedures for remaining study visits as shown in the SoA, with exceptions as noted in this section.

Participants who were assigned to the placebo group and permanently discontinued study intervention in the Double-Blind Treatment Period (that is, discontinued placebo) but remain in the study for scheduled assessments may qualify to participate in the Open-Label Extension Period.

7.1.1. Reasons for Permanent Discontinuation of Study Intervention

A participant must be permanently discontinued from study intervention for the following reasons.

Participant's request — The participant or participant's designee (for example, legally authorized representative) requests to discontinue study intervention.

Investigator discretion — At the discretion of the investigator or designee for safety, behavioral, compliance, or administrative reasons.

Sponsor's decision — For example, the sponsor determines that the participant did not meet the study enrollment criteria specified in Section 5.

AE or clinically significant new medical condition — The participant develops a TEAE, SAE, clinically significant laboratory value, or other safety reason for which the investigator believes that permanent study intervention discontinuation is appropriate.

Hepatic event or liver test abnormality — The participant has a hepatic event or significantly elevated liver function tests requiring permanent discontinuation of study intervention, as described in Section 8.2.6.3.

SAE associated with ARIA, or development of macrohemorrhage — Treatment with study intervention must be permanently discontinued in participants with either

- an SAE associated with treatment-emergent ARIA-E and/or ARIA-H, or
- a macrohemorrhage.

Note: Treatment with study intervention may also be permanently discontinued in participants with any treatment-emergent ARIA-E and/or ARIA-H at the discretion of the investigator and based on severity of clinical and radiologic findings.

For temporary dosing interruption for ARIA-related reasons, see Section 7.1.2.1.

Severe noncompliance — A participant will be permanently discontinued from study intervention if the participant is severely noncompliant with this protocol, resulting in a safety concern in the judgment of the investigator.

Devices not MRI-compatible — A participant will be permanently discontinued from study intervention and will not have study MRIs if the participant requires a ferromagnetic implant or insertion of a cardiac pacemaker that is not MRI-compatible or if an MRI facility that accepts patients with pacemakers is not accessible.

Prohibited therapeutic agent — A participant will be permanently discontinued from study intervention if the participant requires treatment with a prohibited concomitant therapeutic agent (Section 6.9.2).

Other possible reasons that may lead to permanent discontinuation of study intervention based on investigator discretion:

C-SSRS —Study intervention may be permanently discontinued if a participant

- answered "yes" to Question 4 or Question 5 on the "Suicidal Ideation" portion of the C-SSRS, or
- answered "yes" to any of the suicide-related behaviors on the Suicidal Behavior portion of the C-SSRS.

A psychiatrist or appropriately trained mental health professional may assist in the decision to discontinue the participant from study intervention.

Systemic hypersensitivity reaction — A participant may be permanently discontinued from the study intervention, and the sponsor's designated medical monitor should be notified, if the investigator determines that a systemic hypersensitivity reaction has occurred and is related to administration of study intervention. If the investigator is uncertain about whether a systemic hypersensitivity reaction has occurred and whether discontinuation of study intervention is warranted, the investigator may consult the sponsor's designated medical monitor.

7.1.2. Temporary Discontinuation (Interruption) of Study Intervention

If study intervention is held and then resumed, doses must be administered in the sequence prescribed by this protocol. That is, dosing should be resumed where interrupted in the schedule so that the dosing titration sequence is in the order specified by the SoA. If a dose hold occurs between the first and fifth dose, additional on-site visits may be needed to accommodate dose administration by or under the supervision of study site personnel (Section 6.1.1). Other dose adjustments are not permitted.

The following steps must be followed when resuming the study intervention following temporary discontinuation for any reason:

- complete MRIs per the SoA
- resume dosing as per the sequence described in this protocol (Section 6.1), and
- ensure MRIs (unscheduled) are conducted within 7 days prior to the second, third, fourth, fifth, and seventh doses.

Participants who have interruptions in study intervention should attempt to complete the entire dosing regimen within the treatment period. If needed, dosing for the Double-Blind Treatment Period can be extended into the Double-Blind Observation Period to allow additional time to complete the dosing regimen. Any temporary dose interruption in the Open-Label Extension Period should not result in scheduling dosing after end of that period (Visit 110).

The following subsections describe some reasons for temporary discontinuation (interruption) of study intervention.

7.1.2.1. Due to ARIA

Criteria for temporary discontinuation of study intervention

The development of ARIA-E and/or ARIA-H (microhemorrhages and/or cortical superficial siderosis) are expected events and have occurred in participants treated with remternetug.

Study intervention should be temporarily discontinued if a participant has

- clinical symptoms associated with ARIA confirmed by central MRI,
- moderate or severe ARIA-E confirmed by central MRI, or
- recurrent ARIA-E confirmed by central MRI.

The investigator may temporarily discontinue study intervention at any time if the participant develops treatment-emergent ARIA-E or ARIA-H.

If dosing is interrupted due to ARIA or any other reason, continue to perform other study procedures per the SoA.

Additional guidance regarding criteria for temporarily interrupting study intervention due to ARIA can be found in the Manual of Operations.

Monitoring during temporary discontinuation

Follow participants with serial MRIs every 4 to 6 weeks, using fluid attenuation inversion recovery (FLAIR) and T2* gradient-recall echo, to monitor resolution and/or stabilization of ARIA.

There is no defined time duration for this monitoring period while off study intervention and will be based on clinical judgment.

See Section 6.9.1 for possible treatment for symptomatic ARIA-E.

Reinitiating study intervention

If a participant has resolution of non-serious ARIA-E, stabilization of non-serious ARIA-H, and the resolution of any associated symptoms, the study intervention may be resumed at the investigator's discretion.

See Section 7.1.2 for more information regarding reinitiating study intervention following temporary discontinuation.

See the Manual of Operations for further details.

7.1.2.2. Due to Reasons Other Than ARIA

Some reasons for temporary discontinuation of study intervention (interruption) include, but are not limited to, these:

- to evaluate a hepatic event or liver test abnormality (Section 8.2.6.3)
- to evaluate the impact of study intervention on any other AE
- to accommodate a participant's hospitalization or other personal circumstance, or
- to address exceptional study circumstances (Appendix 9, Section 10.9).

If temporary dose interruption is due to an AE, it should be reported in the CRF. Temporary treatment discontinuation and restarting should be documented.

See Section 7.1.2 for more information regarding reinitiating study intervention following temporary discontinuation.

7.2. Participant Discontinuation/Withdrawal from the Study

Discontinuation is expected to be uncommon.

In the unlikely event that the participant becomes pregnant, the participant will be permanently discontinued from the study (Section 8.3.2.1).

A participant may be permanently discontinued from the study if the investigator, site personnel performing assessments, or participant becomes unblinded (Section 6.4).

In addition, a participant may withdraw from the study

- at any time at the participant's own request for any reason or without providing any reason
- at the request of the participant's designee, for example, legally authorized representative
- at the discretion of the investigator for safety, behavioral, compliance, or administrative reasons, or
- if enrolled in any other clinical study involving an investigational product, or if enrolled in any other type of medical research judged not to be scientifically or medically compatible with this study.

The Early Termination visit (ETa) and posttreatment follow-up visit (V801) should only be conducted if a participant has received at least 1 dose of study intervention.

If the participant has not already discontinued the study intervention, the participant will be permanently discontinued from the study intervention at the time of the decision to discontinue the study.

If the participant withdraws consent for disclosure of future information, the sponsor may retain and continue to use any data collected before such a withdrawal of consent. If a participant withdraws from the study, the participant may request destruction of any samples taken and not tested, and the investigator must document this in the site study records.

7.3. Lost to Follow-up

A participant will be considered lost to follow-up if they repeatedly fail to return for scheduled visits and are unable to be contacted by the study site. Site personnel or designee are expected to make diligent attempts to contact participants who fail to return for a scheduled visit or were otherwise unable to be followed up by the site.

Site personnel, or an independent third party, may attempt to collect the vital status of the participant within legal and ethical boundaries for all participants who have received at least 1 dose of study intervention. Public sources may be searched for vital status information. If the participant is determined to be deceased, this will be documented and the participant will not be considered lost to follow-up. Sponsor personnel will not be involved in any attempts to collect vital status information.

Approved on 29 Aug 2025 GMT

8. Study Assessments and Procedures

Timing of assessments and procedures

Study procedures and their timing are summarized in the SoA.

Adherence to the study design requirements, including those specified in the SoA, is essential and required for study conduct.

8.1. Efficacy Assessments

Each visit that includes clinical outcome assessments within the SoA, comprises:

- rater-administered assessments, and/or
- self-administered assessments, proctored by study personnel or site staff.

Rater- and self-administered assessments can be conducted either on the same day or on separate days, provided that the rater-administered session is completed first. The self-administered session should occur within 7 days after the rater-administered session if done on separate days.

Central raters and/or site staff will administer the respective assessments using an eCOA tablet. To facilitate central monitoring of rater-administered scales, the audio voice and/or video recordings of the rater's questions and the participant's and study partner's responses will also be collected during administration of the cognitive and functional testing on the following measures, where local guidelines permit: MMSE, CDR, MoCA, ISLT, and Category fluency.

Clinical outcomes assessments for each participant should be performed at approximately the same time on each day, whenever possible, to reduce potential variability.

When administered on the same day, clinical outcomes assessments should be performed before medical procedures that could be stressful for the participant (for example, blood draws). Note that procedures (for example, MRI) can be conducted on other days within the visit window.

8.1.1. Clinical Dementia Rating (CDR)

The CDR (Hughes et al. 1982; Morris 1993) is a global assessment tool that can be used to effectively evaluate both cognition and function. The tool was initially developed to measure dementia severity and covers 6 categories or "boxes":

- memory
- orientation
- judgment and problem solving
- community affairs
- home and hobbies, and
- personal care.

The CDR-GS, calculated using an algorithm, range from 0 (no dementia) to 3 (severe dementia) while CDR-SB scores, calculated by adding the box scores, range from 0 to 18, with higher scores indicative of more impairment. Scoring is determined by a clinician through a semistructured and in-depth interview with both the affected individual and their study partner. This scale demonstrates acceptable psychometric characteristics (Coley et al. 2011; Cedarbaum

et al. 2013) and has been shown to be sensitive enough to detect disease progression, even in populations with less advanced clinical disease (Williams et al. 2013; Wessels et al. 2015).

CDR must be administered to the study partner prior to the participant and interview must be conducted separately.

8.1.2. Montreal Cognitive Assessment (MoCA)

The MoCA is a brief instrument used to assess cognitive function (Nasreddine et al. 2005). The instrument consists of 13 tasks organized into 8 cognitive domains, including visuospatial/executive function, naming, memory, attention, language, abstraction, delayed recall, and orientation. A total score is generated by summing scores across the 8 domains. The range for the total MoCA score is 0 to 30, with lower scores indicating greater level of impairment.

8.1.3. Category Fluency - Language, Executive Function, Semantic Memory

Category Fluency (Delis et al. 2001) is a measure of language, executive function, and semantic memory that involves planning, organization, and cognitive flexibility. It is a reliable measure of cognitive decline in MCI and AD and has been shown to improve the sensitivity of a preclinical Alzheimer's cognitive composite (Papp et al. 2016, 2017).

The participant is asked to generate as many words as possible in 60 seconds that belong to a given semantic category (for example, animals). The category fluency total score reflects the sum of correct responses generated across all categories presented, with higher values indicating better performance.

8.1.4. Continuous Paired Associate Learning (CPAL) - Visual Episodic Memory

The CPAL measures visual memory using a paired associative learning paradigm (Harel et al. 2011; O'Donnell et al. 2011). In the CPAL version used in this study, the participant must learn and remember a set of 6 patterns hidden beneath 6 different marked spatial locations arranged peripherally around a central location. The CPAL consists of an encoding phase and learning phase.

Encoding phase: In this phase, participants see all the to-be-learned patterns in their assigned spatial location. Each to-be-learned patterns is presented, one at a time in the central location in a random order. With each presentation, the participant must indicate the identical pattern in its peripheral location. Once each of the patterns in the peripheral spatial locations has been matched correctly to that presented in the central location, all information is removed from the marked peripheral locations, and the learning phase of the CPAL commences.

Learning phase: In this phase, the participant is instructed "In what locations do these patterns belong?" One of the 6 to-be-learned patterns is presented in the center location and the participant is required to indicate the marked peripheral location to which they had matched this shape. If they select an incorrect location, an error signal is given, and they must then select a different peripheral location. This trial-and-error process continues until the shape has been placed in its correct peripheral location. The next shape is then shown in the central location and this trial-and-error process is repeated. This continues until all 6 shapes have been placed in their correct marked peripheral locations, and this is classified as 1 learning trial.

Participants complete 6 learning trials on the CPAL with the order of presentation of the to-be-learned patterns randomized between trials. The main performance measure for the CPAL is the number of errors made while learning the 6 pattern-location associations over the 6 trials.

Adults with dementia show very large increases in the number of CPAL errors even when less associations must be learned. In MCI, impairment in CPAL performance was equivalent to that observed for verbal list learning and paragraph recall tests (Harel et al. 2011; Lim et al. 2013). In preclinical AD, over 36 months, amyloid positive individuals compared with amyloid negative individuals showed no practice effect on the CPAL. The absence of benefit from repeated exposure over time was associated with increased amyloid burden (Baker et al. 2019).

8.1.5. International Shopping List Test (ISLT) - Verbal Episodic Memory

The ISLT is a measure of verbal learning and memory, developed specifically for use in people from different cultural and linguistic backgrounds (Lim et al. 2009, 2012a). The ISLT is a 4-trial (3 learning trials and 1 delayed recall), 12-word verbal list learning test that requires the individual to remember a list of items to obtain at a local store.

The verbal stimuli for the ISLT have been selected from food items acknowledged through surveys to be easy to obtain locally in stores, markets, or supermarkets of the cultural group of interest. Because of this, the ISLT has face validity and measures an ability that is part of the daily or regular activities of most people.

In AD dementia and MCI, performance on the ISLT was characterized by reduced number of total words recalled, slower rates of acquisition, impaired primacy with relatively intact recency, and impaired delayed recall when compared to healthy-matched controls (Lim et al. 2012b; Thompson et al. 2011). A qualitatively similar but quantitatively less severe pattern of abnormal performance is observed in adults with prodromal AD (Lim et al. 2012b).

In clinical trials, the main performance measure for the ISLT is the total words recalled correctly across the 3 learning trials and the total words recalled on the delayed recall trial.

8.1.6. International Daily Symbol Substitution Test-Medicines (iDSSTm) - Complex Attention Memory

The DSST paradigm has been well validated as an assessment of complex attention and simple executive function using both (Jaeger 2018):

- paper-and-pencil cognitive assessment, and
- computer-based administration and scoring methods

Different versions of the DSST paradigm have been shown to be important in understanding cognitive decline in preclinical AD (Papp et al. 2020). Because the symbols in conventional DSST paradigms are derived from English language keyboards, there is potential for cultural and linguistic bias in the results of the test when it is applied in some language groups even after translation.

The iDSSTm is a computer-administered digit symbol substitution test and was developed to provide an assessment of complex attention or simple executive function in older adults from different cultural and linguistic backgrounds by utilizing images of different types of medicines

(that is, capsules, tablets) as symbols and numeric calendar dates (for example, February 1 to February 9) as the numbers (Williamson et al. 2022).

On the iDSSTm, participants are shown a key/legend that indicates the associations between 9 medicines, which differ from one another according to a combination of shape and color, and 9 different calendar dates ordered from 1 to 9 (left to right) and denoted by an abbreviated month (that is, Feb for February). On any administration of the iDSSTm, the medicine stimuli are drawn at random from a pool of 108 different images and paired randomly with the different calendar dates. This use of multiple medicine stimuli in randomized recombination with the 9 dates minimizes improvements in performance due to repeated administration, even over short retest intervals (that is, minutes or hours).

In an administration of the iDSSTm,

- the reference key/legend to the association is presented at the top of the display
- a matrix of empty date boxes with a different calendar date is presented in the middle of the display, and
- 9 images of the different medicines are presented at the bottom of the display.

The participant is trained to consider each empty date box, one at a time, relevant to the date-medicine associations at the top of the display and then select from the bottom images the medicine associated with the empty date box. If the participant selects an incorrect medicine, the date box remains empty, and the participant is cued to select a different medicine.

There is a brief set of training trials at the start of each test, and once the participant demonstrates that they understand the instructions, the test begins, and the participant must continue to make the correct associations as fast as they can for 120 seconds.

The main performance measure for the iDSSTm is the number of correct associations between date and medicine made within the 120 seconds. In healthy older adults and in adults with AD dementia, performance on the iDSSTm correlates strongly with performance on paper and pencil DSST measures (that is, the coding test from the Wechsler Adult Intelligence Test, WAIS) (Williamson et al. 2022).

8.1.7. Quick Dementia Rating System (QDRS)

The QDRS is a brief (3 to 5 minutes), self-reported (QDRS-Participant) and observer-reported (QDRS-Study Partner) scale evaluating

- all 6 domains included in the standard CDR scale, which are
    - memory
    - orientation
    - problem solving
    - community affairs
    - home and hobbies, and
    - personal care, and
- 4 additional domains, which are
    - behavior and personality changes
    - language and communication abilities
    - mood, and

- attention and concentration.

QDRS-derived CDR-GS and CDR-SB scores are calculated based on reported impairment on the 6 standard CDR domains. The QDRS total score, calculated as the sum of all 10 individual QDRS domain scores, ranges from 0 to 30, with higher scores indicating greater impairment (Galvin 2015).

Previous cross-sectional data demonstrated strong correspondence between QDRS-derived and traditional CDR-GS and CDR-SB scores and good agreement with established measures of cognition, function, and behavior (Galvin 2015; Galvin et al. 2020; Duff et al. 2022). The QDRS is completed independently, without the need for a trained rater or clinician.

8.1.8. Cognitive Function Index (CFI) – Participant and Study Partner

The CFI is a modified version of the Mail-in Cognitive Function Screening Instrument (Walsh et al. 2006; Amariglio et al. 2015, 2021), developed by the Alzheimer's Disease Cooperative Study (ADCS). The 2 versions of the scale, the Participant CFI and Study Partner CFI, include 15 independently rated items assessing subjective patient- and observer-reported changes in cognitive and functional abilities over the past year, respectively.

8.1.9. Mild Behavioral Impairment Checklist (MBI-C)

The MBI-C (Ismail et al. 2017) provides standardized assessment and monitoring of neuropsychiatric symptoms in preclinical and prodromal AD. Developed in alignment with research diagnostic criteria for mild behavioral impairment (Ismail et al. 2016), the MBI-C provides sensitivity to and classification of patients based on the emergence of sustained and impactful neuropsychiatric symptoms that may precede the emergence or progression of disease-related impairments in cognition and function (Ismail et al. 2017, 2021, 2023; Wise et al. 2019; Martin and Velayudhan 2020; Jin P et al. 2023).

The MBI-C includes 34-items assessing neuropsychiatric symptoms across 5 domains, including:

- decreased motivation
- emotional dysregulation
- impulse control
- social inappropriateness, and
- abnormal perception or thought content.

The MBI-C is implemented as an observer-reported outcome completed by the participant's study partner. For each item, the study partner provides a yes/no response indicating the presence or absence of the symptom described. Endorsed items are further rated as "mild" "moderate" or "severe" based on observed severity. The MBI-C total score range is 0 to 102, with higher scores indicating greater impairment.

8.1.10. Mini-Mental State Examination (MMSE)

The MMSE is a brief instrument used to assess cognitive function in participants (Folstein et al. 1975).

The instrument measures orientation, memory, and attention; the ability of the participant to name objects; follow verbal and written commands; write a sentence; and copy figures. The

range for the total MMSE score is 0 to 30, with lower scores indicating greater level of impairment.

8.1.11. Functional Activities Questionnaire (FAQ)

The FAQ is a brief, 10-item study partner-reported outcome measure assessing observed or perceived ability in performing instrumental everyday activities, typically impacted early in the course of AD (Pfeffer et al. 1982; González et al. 2022). Examples of activities examined include managing finances (paying bills, doing taxes), shopping, meal preparation, awareness of current events, and travel. A total FAQ score of ≥6 has shown optimal sensitivity and specificity in distinguishing between MCI and mild AD dementia (Teng et al. 2010).

The FAQ total score ranges from 0 to 30, with higher scores indicating greater impairment.

8.1.12. ADCS Activities of Daily Living – Prevention Instrument, 18-item (ADCS-ADL-PI) Participant and Study Partner

The ADCS-ADL-PI is a self-reported and study-partner–reported measure assessing daily functioning across complex instrumental activities of daily living, also called iADLs (Galasko et al. 2006). The original instrument, developed for use in AD prevention trials, includes 15-items assessing the participant's performance on tasks, such as managing finances, planning/organization, travel, and meal planning, among others (Galasko et al. 2006).

An 18-item version of the ADCS-ADL-PI (Marshall et al. 2020) includes an additional 3 items (items 16–18) assessing the use of modern technologies such as smartphones, computers, tablets, and e-readers. Participants and their study partners independently rate the participant's level of ability. Total scores for the 18-item ADCS-ADL-PI range from 0 to 54, with higher scores indicating better performance.

8.2. Safety Assessments

Timing and order of safety assessments

The planned time points for safety assessments are provided in the SoA.

If multiple safety assessments are scheduled for the same visit, the following order of activities is recommended:

1. vital signs and ECG, followed by
2. other safety assessments, including physical examinations and nonleading (spontaneous) AE collection, followed by C-SSRS, and finally
3. blood sample collections.

Data collection and reporting

The AE data collection and reporting requirements are described in Section 8.3 and Appendix 3, Section 10.3. Additional requirements regarding AESIs are described in Section 8.3.3.

Any clinically significant findings that result in a diagnosis and that occur after the participant receives the first dose of study intervention should be reported to the sponsor or its designee as an AE via CRF.

Safety monitoring

The principal investigator will monitor the safety data throughout the study. Immediate safety concerns should be discussed with the sponsor immediately upon occurrence or awareness to determine if the participant should continue or discontinue study intervention.

8.2.1. Height and Weight

Height and weight, which may be collected via a participant's remote report to the investigator, will be collected according to the SoA.

8.2.2. Electrocardiograms

For each participant, a 12-lead digital ECG will be collected during screening according to the SoA. Participants must be supine for approximately 5 to 10 minutes before ECG collection and remain supine but awake during ECG collection. ECGs must be recorded before collecting any blood samples.

ECGs (single) may be obtained at additional times, when deemed clinically necessary.

ECGs will be interpreted by the investigator (a physician or qualified designee) at the site as soon after the time of ECG collection as practical.

The investigator or qualified designee must document their review of the ECG printed at the time of collection.

Any new clinically relevant finding should be reported as an AE.

8.2.3. Physical Examination

Physical examinations will be performed according to the SoA.

The complete physical examination will include assessment of

- general appearances
- head and neck
- skin
- lymph nodes
- thyroid
- abdomen (bowel sounds, liver, and spleen palpation)
- back (costovertebral angle tenderness), and
- musculoskeletal, cardiovascular, and respiratory systems.

The neurological examination will include a thorough assessment of

- cranial nerves
- sensory and motor systems
- gait
- balance
- coordination, and
- reflexes.

Symptom-directed physical examinations (including neurological examinations) may be performed at other study visits at the discretion of the investigator. Any clinically significant changes from baseline should be noted on the AE CRF.

If necessary, a neurologist may be consulted in the event of significant new findings.

If a potentially clinically meaningful change in an MRI is noted during the study, an additional neurological examination should be performed as soon as possible, along with any other medical follow-up deemed necessary by the investigator.

8.2.4. Vital Signs

Vital sign measurements (blood pressure, pulse rate, and body temperature) will be conducted according to the SoA. If warranted, additional vital signs may be measured.

Blood pressure and pulse rate should be measured after approximately 5 minutes in the sitting position.

Unscheduled orthostatic vital signs may be assessed, if possible, during any AE of dizziness or posture-induced symptoms.

8.2.5. Clinical Safety Laboratory Tests

See Appendix 2, Section 10.2, for the list of clinical laboratory tests to be performed and the SoA for the timing and frequency.

All protocol-required laboratory assessments, as defined in Appendix 2, Section 10.2, must be conducted in accordance with the SoA, standard collection requirements, and laboratory manual.

If laboratory values from non-protocol–specified laboratory assessments performed at an investigator-designated local laboratory require a change in participant management or are considered clinically significant by the investigator, for example, SAE or AE or dose modification, then report the information as an AE.

Reviewing and recording test results

The investigator must review the laboratory results, document this review, and report any clinically relevant changes occurring during the study as an AE. The laboratory results must be retained with source documents unless a Source Document Agreement or comparable document cites an electronic location that accommodates the expected retention duration.

Repeat testing after clinically significant abnormal findings

Clinically significant abnormal laboratory findings are those which are not associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.

All laboratory tests with values considered clinically significantly abnormal during participation in the study or within 20 weeks after the last dose of study intervention should be repeated until the values return to normal or baseline or are no longer considered clinically significant by the investigator or medical monitor.

If such values do not return to normal/baseline within a period of time judged reasonable by the investigator, the etiology should be identified and the sponsor notified.

8.2.6. Hepatic Safety Monitoring, Evaluation, and Criteria for Study Drug Interruption or Discontinuation

These tables summarize actions to take based on abnormal hepatic laboratory or clinical changes.

Participants with normal or near normal baseline (ALT, AST, or ALP <1.5x ULN)

If this laboratory value is observed…	Then…		
	Initiate or continue close hepatic monitoring	Initiate comprehensive evaluation	Interrupt or discontinue study drug
ALT or AST ≥3x ULN	X		
ALP ≥2x ULN	X		
TBL ≥2x ULN[a]	X		
ALT or AST ≥5x ULN	X	X	
ALP ≥2.5x ULN	X	X	
ALT or AST ≥3x ULN with hepatic signs or symptoms[b]	X	X	X
ALT or AST ≥5x ULN for more than 2 weeks	X	X	X
ALT or AST ≥8x ULN	X	X	X
ALT or AST ≥3x ULN and TBL ≥2x ULN[a] or INR ≥1.5	X	X	X
ALP ≥3x ULN	X	X	X
ALP ≥2.5x ULN and TBL ≥2x ULN[a]	X	X	X
ALP ≥2.5x ULN with hepatic signs or symptoms[b]	X	X	X

[a] In participants with Gilbert's syndrome, the threshold for TBL will be TBL≥3x ULN, if baseline is <1.5x ULN, and TBL≥2x baseline, if baseline is ≥1.5x ULN.

[b] Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

Participants with elevated baseline (ALT, AST, or ALP ≥1.5x ULN)

If this laboratory value is observed…	Then…		
	Initiate or continue close hepatic monitoring	Initiate comprehensive evaluation	Interrupt or discontinue study drug
ALT or AST ≥2x baseline	X		
ALP ≥2x baseline	X		
TBL ≥2x ULN[a]	X		
ALT or AST ≥3x baseline or ≥250 U/L (whichever occurs first)	X	X	
ALP ≥2.5x baseline	X	X	
ALT or AST ≥2x baseline or ≥250 U/L (whichever occurs first) with hepatic signs or symptoms[b]	X	X	X
ALT or AST ≥3x baseline or ≥250 U/L (whichever occurs first) for more than 2 weeks	X	X	X

If this laboratory value is observed…	Then…		
	Initiate or continue close hepatic monitoring	**Initiate comprehensive evaluation**	**Interrupt or discontinue study drug**
ALT or AST ≥4x baseline or ≥400 U/L (whichever occurs first)	X	X	X
ALT or AST ≥2x baseline or ≥250 U/L (whichever occurs first) and TBL ≥2x ULN[a] or INR ≥1.5	X	X	X
ALP ≥3x baseline	X	X	X
ALP ≥2.5x baseline and TBL ≥2x ULN[a]	X	X	X
ALP ≥2.5x baseline with hepatic signs or symptoms[b]	X	X	X

[a] In participants with Gilbert's syndrome, the threshold for TBL will be TBL≥3x ULN, if baseline is <1.5x ULN, and TBL≥2x baseline, if baseline is ≥1.5x ULN.

[b] Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

8.2.6.1. Close Hepatic Monitoring

If a participant develops any one of these changes, initiate close hepatic monitoring:

Participants with normal or near normal baseline (ALT, AST, or ALP <1.5x ULN)	Participants with elevated baseline (ALT, AST, or ALP ≥1.5x ULN)
ALT or AST ≥3x ULN **or**	ALT or AST ≥2x baseline
ALP ≥2x ULN **or**	ALP ≥2x baseline
TBL ≥2x ULN[a]	TBL ≥2x ULN[a]

[a] In participants with Gilbert's syndrome, the threshold for TBL will be TBL≥3x ULN, if baseline is <1.5x ULN, and TBL≥2x baseline, if baseline is ≥1.5x ULN.

Close hepatic monitoring should include these actions:

- Laboratory tests (Appendix 5, Section 10.5), including ALT, AST, ALP, TBL, direct bilirubin, gamma-glutamyltransferase, creatine kinase, and complete blood count with differential, should be checked within 48 to 72 hours of the detection of elevated liver tests to confirm the abnormality and to determine if it is increasing or decreasing.
- If the abnormality persists, clinical and laboratory monitoring should continue at a frequency of 2 to 3 times weekly until levels normalize or return to approximate baseline values. Subsequently, the frequency of monitoring may be lowered to once every 1 to 2 weeks, if the participant's clinical condition and laboratory results stabilize.

- In addition to laboratory tests, basic evaluation for possible causes of abnormal liver tests should be initiated by the investigator in consultation with the sponsor's designated medical monitor. At a minimum, this evaluation should include physical examination and a thorough medical history, including current symptoms, recent illnesses (for example, heart failure, systemic infection, hypotension, or seizures), recent travel, concomitant medications (including over-the-counter), herbal and dietary supplements, history of alcohol drinking, and other substance abuse.

8.2.6.2. Comprehensive Hepatic Evaluation

If a participant develops any one of the following laboratory or clinical changes, initiate a comprehensive hepatic evaluation:

Participants with normal or near normal baseline (ALT, AST, or ALP <1.5x ULN)	**Participants with elevated baseline (ALT, AST, or ALP ≥1.5x ULN)**
ALT or AST ≥5x ULN **or**	ALT or AST ≥3x baseline or ≥250 U/L (whichever occurs first) **or**
ALP ≥2.5x ULN **or**	ALP ≥2.5x baseline **or**
ALT or AST ≥3x ULN with hepatic signs or symptoms[a] **or**	ALT or AST ≥2x baseline or ≥250 U/L (whichever occurs first) with hepatic signs or symptoms[a] **or**
ALT or AST ≥3x ULN and TBL ≥2x ULN[b] or INR ≥1.5	ALT or AST ≥2x baseline or ≥250 U/L (whichever occurs first) and TBL ≥2x ULN[b] or INR ≥1.5

[a] Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

[b] In participants with Gilbert's syndrome, the threshold for TBL will be TBL ≥3x ULN if baseline is <1.5x ULN, and TBL ≥2x baseline if baseline is ≥1.5x ULN.

Comprehensive hepatic evaluation should include these actions:

- At a minimum, comprehensive hepatic evaluation should include physical examination and a thorough medical history, as outlined above, as well as tests for PT-INR; tests for viral hepatitis A, B, C, and E; tests for autoimmune hepatitis; and an abdominal imaging study (for example, ultrasound or CT scan).
- Based on the participant's history and initial results, further testing should be considered in consultation with the sponsor's designated medical monitor, including tests for hepatitis D virus, cytomegalovirus, Epstein-Barr virus, acetaminophen levels, acetaminophen protein adducts, urine toxicology screen, Wilson's disease, blood alcohol levels, urinary ethyl glucuronide, and blood phosphatidylethanol.
- Based on the circumstances and the investigator's assessment of the participant's clinical condition, the investigator should consider referring the participant for a hepatologist or gastroenterologist consultation, and additional tests including magnetic resonance cholangiopancreatography, endoscopic retrograde cholangiopancreatography, cardiac echocardiogram, or a liver biopsy.
- Clinical and laboratory monitoring should continue at a frequency of 1 to 3 times weekly until levels normalize or return to approximate baseline values.
- All the medical information and tests results related to the hepatic monitoring and comprehensive hepatic evaluation should be collected and recorded in a hepatic safety CRF.

8.2.6.3. Study Drug Interruption or Discontinuation

If a participant develops any one of the following laboratory or clinical changes, interrupt the study drug and continue close monitoring and comprehensive hepatic evaluation as described in Sections 8.2.6.1 and 8.2.6.2.

Participants with normal or near normal baseline (ALT, AST, or ALP <1.5x ULN)	**Participants with elevated baseline (ALT, AST, or ALP ≥1.5x ULN)**
ALT or AST ≥3x ULN with hepatic signs or symptoms[a] **or**	ALT or AST ≥2x baseline or ≥250 U/L (whichever occurs first) with hepatic signs or symptoms[a] **or**
ALT or AST ≥5x ULN for more than 2 weeks **or**	ALT or AST ≥3x baseline or ≥250 U/L (whichever occurs first) for more than 2 weeks **or**
ALT or AST ≥8x ULN **or**	ALT or AST ≥4x baseline or ≥400 U/L (whichever occurs first) **or**
ALT or AST ≥3x ULN and TBL ≥2x ULN[b] or INR ≥1.5 **or**	ALT or AST ≥2x baseline or ≥250 U/L (whichever occurs first) and TBL ≥2x ULN[b] **or**
ALP ≥3x ULN **or**	ALP ≥3x baseline **or**
ALP ≥2.5x ULN and TBL ≥2x ULN[b] **or**	ALP ≥2.5x baseline and TBL ≥2x ULN[b] **or**
ALP ≥2.5x ULN with hepatic signs or symptoms[a]	ALP ≥2.5x baseline with hepatic signs or symptoms[a]

[a] Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

[b] In participants with Gilbert's syndrome, the threshold for TBL will be TBL ≥3x ULN if baseline is <1.5x ULN, and TBL ≥2x baseline if baseline is ≥1.5x ULN.

Interruption or discontinuation of study drug should include these actions:

- While the participant is not receiving the study drug, clinical and laboratory monitoring should continue at a frequency of 1 to 3 times weekly until liver tests normalize or return to approximate baseline values.
- If the hepatic event continues past the anticipated end of the study (that is, data lock) the investigator should consult with the sponsor's designated medical monitor to determine the need for further data collection beyond the end date of the study (that is, data lock date).
- All the medical information and tests results related to the close hepatic monitoring and comprehensive hepatic evaluation should be collected and recorded in a hepatic safety CRF.
- Resumption of the study drug after interruption for a hepatic reason can be considered only in consultation with the sponsor's designated medical monitor and only if the liver test results returned to near baseline and if a self-limited, non-study-drug etiology is identified. Otherwise, the study drug should be permanently discontinued.

8.2.7. Suicidal Ideation and Behavior Risk Monitoring

Participants being treated with study intervention should be monitored appropriately and observed closely for SIB or any other unusual changes in behavior, especially at the beginning and end of the course of intervention, or at the time of dose changes, either increases or decreases. Participants who experience signs of SIB should undergo a risk assessment. All factors contributing to SIB should be evaluated and consideration should be given to discontinuation of the study intervention.

When informed consent has been given, families and partners of participants being treated with study intervention should be alerted about the need to monitor participants for the emergence of unusual changes in behavior, as well as the emergence of SIB and to report such symptoms immediately to the study investigator. Baseline assessment of SIB and intervention-emergent SIB will be monitored during this study using the C-SSRS, according to the SoA.

8.2.7.1. C-SSRS

The C-SSRS is a scale that captures the occurrence, severity, and frequency of suicidal ideation and behavior during the assessment period via a semi-structured interview by a trained rater.

For this study, the full version of the C-SSRS will be used.

The C-SSRS data will be collected using an eCOA tablet.

Timing of collection and AE monitoring

Nonleading (unsolicited) AE collection should occur prior to the collection of the C-SSRS.

Report on the AE form a suicide-related event discovered during collection of the C-SSRS only if the event

- is an AE that leads to discontinuation, or
- is an SAE (follow standard procedures for reporting SAEs).

8.2.8. Magnetic Resonance Imaging

MRI of the brain will be performed according to the SoA and as clinically indicated.

Unscheduled MRIs may be performed at the discretion of investigator. The “Unscheduled MRI” CRF should be completed to provide rationale for all unscheduled MRIs. MRIs will be used to check for evidence of ARIA-H or ARIA-E, and other clinically relevant safety findings, including those which might exclude a participant from the study and to evaluate longitudinal MRI changes.

The MRI scans will be reviewed by the investigator or qualified designee for immediate participant management.

MRI scans are read locally and are sent for analysis to a centralized MRI vendor designated by Lilly. The centralized MRI vendor will determine the participant’s final MRI-based eligibility, and the Visit 1 screening MRI results will be reported to the investigator as “does” or “does not” meet entry criteria based on MRI eligibility (Section 5.2). Results of centrally read MRIs regarding participant care or safety will also be reported back to the investigator.

Specific analyses of the scans, including assessments of ARIA-H and ARIA-E, will be interpreted by the centralized MRI vendor for data analysis and report-writing purposes.

At visits where both MRI and dosing are scheduled to occur according to the SoA, the MRI must be reviewed by the investigator within 7 days prior to dosing. The decision to administer the dose should be based on the central read; however, the local read may be used for dosing decisions if the central read is still pending. If the central read differs, dosing decisions based on the locally read MRI will not be considered a protocol deviation. For the purposes of data

collection and analysis, if there is a discrepancy between the locally and centrally read MRIs, the centrally read MRI takes precedence.

8.3. Adverse Events, Serious Adverse Events, and Product Complaints

The definitions of the following events can be found in Appendix 3, Section 10.3:

- AEs
- SAEs, and
- PCs.

These events will be reported by the participant, or, when appropriate, by a caregiver, study partner, surrogate, or the participant's legally authorized representative.

The investigator and any qualified designees are responsible for detecting, documenting, and recording events that meet these definitions and remain responsible for following up events that are serious, considered related to the study intervention or study procedures, or that caused the participant to discontinue the study intervention or the study (Section 7).

Care will be taken not to introduce bias when detecting events. Open-ended and nonleading verbal questioning of the participant is the preferred method to inquire about event occurrences.

After the initial report, the investigator is required to proactively follow each participant at subsequent visits or contacts. All SAEs and AESIs (as defined in Section 8.3.3) will be followed until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.3).

For PCs, the investigator is responsible for ensuring that follow-up includes any supplemental investigations as indicated to elucidate the nature or causality. Further information on follow-up procedures is provided in Appendix 3, Section 10.3.4.

8.3.1. Timing and Mechanism for Collecting Events

This table describes the timing, deadlines, and mechanism for collecting events.

Event	Collection Start	Collection Stop	Timing for Reporting to Sponsor or Designee	Mechanism for Reporting	Back-up Method of Reporting
Adverse Event					
AE	Signing of the ICF	Participation in study has ended	As soon as possible upon site awareness	AE CRF	N/A
Serious Adverse Event					
SAE and SAE updates – prior to start of study intervention **and** deemed reasonably possibly related to study procedures	Signing of the ICF	Start of intervention	Within 24 hours of awareness	SAE CRF	SAE paper form
SAE and SAE updates – after start of study intervention	Start of intervention	Participation in study has ended	Within 24 hours of awareness	SAE CRF	SAE paper form
SAE[a] – after participant's study participation has ended **and** the investigator becomes aware	After participant's study participation has ended	N/A	Promptly	SAE paper form	N/A

Event	Collection Start	Collection Stop	Timing for Reporting to Sponsor or Designee	Mechanism for Reporting	Back-up Method of Reporting
Pregnancy					
Pregnancy in participants and in partners of participants	After the start of study intervention	Generally, the follow-up will be no longer than 6 to 8 weeks following the estimated delivery date, or 20 weeks from the last study drug administration, whichever is longer (Section 8.3.2)	Within 24 hours (see Section 8.3.2)	Pregnancy paper form	Pregnancy paper form
Product Complaints					
PC associated with an SAE or might have led to an SAE	Start of study intervention	End of study intervention	Within 24 hours of awareness	PC form	N/A
PC not associated with an SAE	Start of study intervention	End of study intervention	Within 1 business day of awareness	PC form	N/A
Updated PC information	—	—	As soon as possible upon site awareness	Originally completed PC form with all changes signed and dated by the investigator	N/A
PC (if investigator becomes aware)	Participation in study has ended	N/A	Promptly	PC form	

Abbreviations: AE = adverse event; CRF = case report form; ICF = informed consent form; N/A = not applicable; PC = product complaint; SAE = serious adverse event.

[a] These SAEs should not be reported unless the investigator deems them to be possibly related to study treatment or study participation.

8.3.2. Collection of Pregnancy Information

8.3.2.1. Participants Who Become Pregnant

The investigator will collect pregnancy information on any participant who becomes pregnant while participating in this study. The initial information will be recorded on the appropriate form and submitted to the sponsor within 24 hours of learning of a participant's pregnancy.

The participant will be followed to determine the outcome of the pregnancy. The investigator will collect follow-up information on the participant and the neonate and the information will be forwarded to the sponsor. Generally, follow-up will not be required for longer than 6 to 8 weeks beyond the estimated delivery date. Any termination of pregnancy will be reported, regardless of gestational age, fetal status (presence or absence of anomalies) or indication for the procedure.

While pregnancy itself is not considered to be an AE or SAE, any pregnancy complication or elective termination of a pregnancy for medical reasons will be reported as an AE or SAE.

A spontaneous abortion (occurring at <20 weeks gestational age) or still birth (occurring at ≥20 weeks gestational age) is always considered to be an SAE and will be reported as such.

Any poststudy pregnancy related SAE considered reasonably related to the study intervention by the investigator will be reported to the sponsor as described in Section 8.3.1. While the investigator is not obligated to actively seek this information in former study participants, the investigator may learn of an SAE through spontaneous reporting.

Any participant who becomes pregnant while participating in the study will be withdrawn from the study (Section 7.2). If the participant is discontinued from the study, follow the standard discontinuation process and continue directly to the follow-up phase. The follow-up on the pregnancy outcome should continue independent of intervention or study discontinuation.

8.3.2.2. Participants With Partners Who Become Pregnant

When to collect pregnancy information

In most circumstances, the investigator will attempt to collect pregnancy information from a participant's partner who becomes pregnant while the participant is in this study.

After learning about a pregnancy in the partner of a study participant, the investigator

- will obtain a consent to release information from the pregnant partner directly, and
- within 24 hours after obtaining this consent, will record pregnancy information on the appropriate form and submit it to the sponsor.

The partner will be followed to determine the outcome of the pregnancy. Information on the status of the mother and neonate will be forwarded to the sponsor. Generally, the follow-up will be no longer than 6 to 8 weeks after the estimated delivery date. Any termination of the pregnancy will be reported regardless of gestational age, fetal status (presence or absence of anomalies) or indication for the procedure.

When not to collect pregnancy information

It is not necessary to collect information about a pregnancy in the partner of a study participant in these circumstances:

- the partner of the study participant was not exposed to the study intervention, or
- the participant did not contribute the sperm or ova that resulted in the pregnancy.

8.3.3. Adverse Events of Special Interest

Specific safety topics of interest for this study include, but are not limited to, these:

- ARIA-E
- ARIA-H
- cerebral hemorrhage (macrohemorrhage)
- hypersensitivity, immediate and nonimmediate, including anaphylaxis, and
- injection site reactions.

These topics, as well as other topics that may be subsequently determined by the sponsor, will be subject to enhanced surveillance activities. Additionally, these topics will be analyzed for presentation in the CSR in accordance with the SAP.

8.3.3.1. Amyloid-Related Imaging Abnormalities (ARIA-E and ARIA-H) and Macrohemorrhages

While most cases of ARIA are asymptomatic, serious cases have been reported. Additionally, serious cases of cerebral hemorrhage (macrohemorrhage) have also been reported.

If a participant simultaneously develops more than 1 of the symptoms suggestive of ARIA, an unscheduled MRI should be performed. A single symptom suggestive of ARIA may also warrant an MRI. See Section 7.1 for discontinuation of study intervention due to ARIA.

In the event of a finding of ARIA, complete the appropriate “Clinical Events of ARIA” CRF and report ARIA as an AE. Symptoms of ARIA should be entered in the “Clinical Events of ARIA” CRF and should not be listed as AEs.

In the event of a macrohemorrhage or an SAE associated with treatment-emergent ARIA, study intervention should be permanently discontinued (Section 7.1), and the participant should be monitored with unscheduled MRIs at least every 4 to 6 weeks until stable.

See the Manual of Operations for details on managing and reporting ARIA and macrohemorrhage.

8.3.3.2. Hypersensitivity Reactions

Many drugs, including oral agents and biologic agents, carry the risk of systemic hypersensitivity reactions. If such a reaction occurs, additional data should be provided to the sponsor in the designated CRFs.

Sites should have appropriately trained medical staff and appropriate medical equipment available when study participants are receiving study intervention. It is recommended that participants who experience a systemic hypersensitivity reaction be treated per national and international guidelines.

In the case of a suspected systemic hypersensitivity event, additional blood samples should be collected as described in Appendix 2, Section 10.2.2. Laboratory results are provided to the sponsor via the central laboratory.

8.3.3.3. Injection Site Reactions

Symptoms and signs of a local injection site reaction may include erythema, induration, pain, pruritus, and edema.

If an ISR is reported by a participant or site staff, the ISR CRF will be used to capture additional information about this reaction (for example, injection-site pain, degree and area of erythema, induration, pruritus, and edema).

8.4. Pharmacokinetics

At the visits and times specified in the SoA, blood samples will be collected to determine the serum concentrations of remternetug. Instructions for the collection and handling of blood samples will be provided by the sponsor. The actual date and time (24-hour clock time) of each sampling will be recorded.

Drug concentration information that may unblind the study will not be reported to investigative sites or blinded personnel until the study has been unblinded.

A maximum of 3 blood samples per participant may be drawn at additional time points during the study, if warranted and agreed upon between both the investigator and sponsor. Instructions for the collection and handling of blood samples will be provided by the sponsor.

During the Double-Blind Treatment Period and the Open-Label Extension Period, if study intervention is permanently discontinued but the participant remains in the study, 1 PK sample should be collected at the soonest scheduled visit regardless of whether PK sample collection is on the SoA at that visit. Dosing dates and times should be collected. Subsequent PK sample collection should follow the SoA.

8.5. Pharmacodynamics

Biomarkers, including plasma P-tau, will be evaluated to assess the effects of remternetug (Section 8.7).

8.6. Genetics

8.6.1. Apolipoprotein E Genotyping

Blood sampling for *APOE* genotyping will be performed as shown in the SoA. *APOE* genotyping is a mandatory part of Study LAKI for all enrolled study participants.

APOE genotyping is a mandatory part of this study. Blood sampling for *APOE* genotyping will be performed as shown in the SoA.

Participants may opt to learn the *APOE* genotype if they are p-tau eligible. Eligible participants will have the option to learn their *APOE* genotype before or during the active treatment period.

At the time of this amendment, neither participants nor investigators will receive genotype results unless active participants sign consent for release of information, or if there is a country-specific law or regulation that requires notification of results. Results may be delivered by the principal investigator or a delegate.

8.6.2. Whole Blood Samples for Pharmacogenetic Research

Collection of sample

Where local regulations and IRB/IEC allow, a blood sample will be collected for DNA analysis from consenting participants, as shown in the SoA.

Use and analysis of DNA

Genetic variation may impact a participant's response to study intervention, susceptibility to, and severity and progression of disease. Variable response to study intervention may be due to genetic determinants that impact drug absorption, distribution, metabolism, and excretion; mechanism of action of the drug; disease etiology; and/or molecular subtype of the disease being treated.

DNA samples will be used for research related to related to remternetug or AD and related diseases. They may also be used to develop tests/assays, including diagnostic tests related to remternetug and/or interventions of this drug class and AD. Genetic research may consist of the analysis of one or more candidate genes or the analysis of genetic markers throughout the genome or analysis of the entire genome (as appropriate). The samples may be analyzed as part of a multi-study assessment of genetic factors involved in the response to remternetug or study interventions of this class to understand study disease or related conditions. The results of genetic analyses may be reported in the CSR or in a separate study summary.

Sample retention and storage

The samples will be retained while research on remternetug or study interventions of this class or indication continues but no longer than indicated in Appendix 1, Section 10.1.12.

The sponsor will store the DNA samples in a secure storage space with adequate measures to protect confidentiality.

8.7. Biomarkers

Screening biomarker consistent with brain amyloid pathology

Plasma phosphorylated tau (P-tau), an investigational in vitro diagnostic device (immunoassay) for identifying the presence of abnormal brain amyloid pathology, will be used to determine a participant's eligibility for this study (Section 5.1).

Biomarker research

Plasma P-tau levels will be assessed longitudinally as a PD biomarker.

Additionally, serum, plasma, and whole-blood RNA will be collected for exploratory biomarker research. These samples, as well as the P-tau and *APOE* samples, may be used for 1 or more of the following study areas:

- research on the drug target
- variable response to remternetug
- mechanisms of action of remternetug
- research methods of remternetug or AD
- validating diagnostic tools or assays related to remternetug or AD
- pathways associated with AD.

For sample collection information, see Appendix 2, Section 10.2, and the SoA.

Sample retention is described in Appendix 1, Section 10.1.12.

8.8. Immunogenicity Assessments

At the visits and times specified in the SoA, venous blood samples will be collected to determine antibody production against remternetug. Antibodies may be further characterized for their ability to neutralize the activity of remternetug. To interpret the results of immunogenicity, a venous blood sample will be collected at the same time points to determine the serum concentrations of remternetug. All samples for immunogenicity should be taken predose when applicable and possible. Treatment-emergent ADA are defined in Section 9.3.6.2.

Immunogenicity will be assessed by a validated assay designed to detect ADA in the presence of remternetug at a laboratory approved by the sponsor. The purpose of retention, the maximum duration of retention, and facility for long-term storage of samples is described in Appendix 1, Section 10.1.12. Samples may also be used for development and control of an immunogenicity assay.

8.9. Medical Resource Utilization and Health Economics

8.9.1. Resource Utilization Inventory - Brief (RUI-B)

The RUI-B will be administered to assess changes associated with the transition from normal aging to dementia in

- health-related resource use (medical care, nonmedical care, and informal care), and
- participant's involvement in volunteer and paid work.

Both participants and their study partners contribute to the reporting of the RUI-B to maximize information obtained. The RUI-B has been used in trials of normal aging (Sano et al. 2006), MCI (Zhu et al. 2013), and dementia (Zhu et al. 2006), and items are included that are relevant to the specific stage of cognitive impairment and dementia. For this trial, a brief version of the inventory will be used (the RUI-B, where B stands for brief), including only those items that are relevant to this stage with a focus on volunteer and paid work and items for medical and nonmedical resources.

8.9.2. Quality of Life in Alzheimer's Disease (QOL-AD)

The QOL-AD is a valid, reliable, and disease-specific measure of quality of life for an AD population (Logsdon et al. 1999, 2002; Thorgrimsen et al. 2003). It includes 13 items, each rated on a 4-point scale. Summing the items provides an overall score to index the participant's quality

of life. The QOL-AD is administered to the participant by a rater and asks the participant to provide ratings on mood, relationships, memory, finances, etc. The participant's study partner also is asked to complete the same measure.

8.9.3. Zarit Burden Interview (ZBI-22)

ZBI is a caregiver-reported instrument used widely to evaluate caregiver burden associated with AD and other diseases, such as congestive heart failure, multiple sclerosis, and cancer. The standard ZBI includes 22 items derived from the original 29-item questionnaire described by Zarit and colleagues (Bédard et al. 2001).

The 22-item ZBI includes questions on caregiver stress, time available for self and impact of caring on the caregiver's social life. Responses are on a 5-point Likert scale (never = 0, nearly always = 4). The total burden is obtained by adding the scores for all the items, with a range of 0 to 88, with higher scores indicating greater burden.

9. Statistical Considerations

The SAP will be finalized prior to unblinding, and it will include a more technical and detailed description of the statistical analyses described in this section. This section is a summary of the planned statistical analyses of the most important endpoints, including primary and key secondary endpoints.

9.1. Statistical Hypotheses

The primary objective is to assess the effect of remternetug versus placebo with respect to clinically meaningful progression in participants with early AD as measured by the time to clinically meaningful progression, defined as an increase from baseline at two consecutive time points either in CDR-SB of 1 or greater or in CDR-GS of 0.5 or greater.

Key secondary hypotheses are that remternetug is superior to placebo with respect to clinical progression in participants with early AD, as measured by

- CDR-SB
- MoCA
- Category Fluency
- CPAL
- ISLT
- iDSSTm
- CFI
- MBI-C, and
- ADCS-ADL-PI.

Cognitive composite score involving a combination of the individual assessments may be analyzed as a secondary endpoint.

The details on the testing schemes for the primary endpoint and key secondary endpoints will be discussed in the SAP for the primary endpoint and key secondary endpoints.

9.1.1. Multiplicity Adjustment

The overall family-wise type I error rate for the primary endpoint of interest will be controlled at a 2-sided level of 5%, or equivalently, a 1-sided level of 2.5%. A graphical approach may be utilized to provide strong control of the study-wise Type I error rate for the key secondary hypotheses at an alpha level of 0.05 (Bretz et al. 2009, 2011). This approach will protect against type I error of falsely rejecting a null hypothesis. Details on the graphical approach and testing strategy will be specified in the SAP prior to unblinding.

9.2. Analyses Sets

This table defines the sets for the purposes of analyses.

Population	Description
Entered	All participants who sign the ICF
Enrolled/ITT	All participants assigned to treatment, regardless of whether they take any doses of study treatment, or if they took the correct treatment. Participants will be analyzed according to the treatment group to which they were assigned.
Efficacy evaluable	For each assessment, the efficacy evaluable population will be defined as all participants with a baseline measurement and at least 1 complete postbaseline measurement for that assessment. Participants will be grouped according to randomized treatment assignment (remternetug or placebo), even if the participant does not take the assigned treatment, does not receive the correct treatment, or otherwise does not follow the protocol.
Open-label extension	If this period is activated, all participants who participated in the open-label extension period and received at least 1 dose of remternetug.
Safety	All participants randomly assigned to study intervention and exposed to at least 1 dose of study intervention
Pharmacokinetic analysis	Same as Safety Population

Abbreviations: ICF = informed consent form; ITT = intention-to-treat.

9.3. Statistical Analyses

9.3.1. General Considerations

Statistical analyses of this study will be the responsibility of the sponsor or its designee.

Unless otherwise noted, all pairwise tests of treatment effects will be conducted at a 2-sided alpha level of 0.05 (or equivalently, a 1-sided 0.025 alpha level); 2-sided CIs will be displayed with a 95% confidence level. All tests of interactions between treatment and other factors will be conducted at an alpha level 0.05.

All efficacy analyses will follow the ITT principle unless otherwise specified. An ITT analysis is an analysis of data by the groups to which participants are assigned by random allocation, even if the participant does not take the assigned treatment, does not receive the correct treatment, or otherwise does not follow the protocol.

Unless otherwise defined, a baseline measure is the last nonmissing observation collected prior to the first administration of study intervention.

For primary and each of the key efficacy analyses, observations collected at nonscheduled visits will not be included in the analyses, unless otherwise specified.

A database lock is expected to occur after the target number of primary outcome events have occurred (Section 4.1). Efficacy and safety analyses will be conducted based on data collected during the double-blind periods of the study (that is, Double-Blind Treatment Period and Double-Blind Observation Period). Data collected during the Open-Label Extension Period (if activated) and Follow-Up periods will be summarized and analyzed separately.

Any changes to the data analysis methods will be described in the SAP prior to unblinding. A protocol amendment will only be required if it changes a principal feature of the protocol conduct impacting sites or participants directly. Any other changes to the data analysis methods described in the protocol, and the justification for making the change, will be described in the SAP and the CSR. Additional exploratory analyses of the data will be conducted as deemed appropriate.

Handling of missing, unused, and spurious data is addressed prospectively in the overall statistical methods described in the protocol and in the SAP, where appropriate. Adjustments to the planned analyses are described in the final CSR.

9.3.2. Primary Endpoint Analysis

The primary analysis will be based on an intent-to-treat principle according to the treatment group to which participants were assigned. The primary outcome is the time to clinically meaningful progression, defined as an increase from baseline at two consecutive time points either in CDR-SB of 1 or greater or in CDR-GS of 0.5 or greater. Time to progression is defined as the time from randomization until the progression event is observed. For statistical analyses, the time of the progression event will be defined as the date of the first of the two consecutive assessments at which the CDR-GS or CDR-SB met progression criteria.

Any additional adjudication to the primary outcome will be prespecified in the SAP prior to unblinding of clinical efficacy data.

The primary efficacy analysis model is a Cox proportional-hazards model. The model includes terms for treatment, *APOE* ε4 genotype, baseline CDR-GS, geographical regions and age at baseline as fixed effects. Additional covariates maybe specified in the SAP. The primary efficacy analysis will be conducted when at least the prespecified number of events has been achieved, as stated in Section 9.5. Additional sensitivity analyses will be prespecified in the SAP prior to unblinding.

9.3.3. Secondary Endpoints Analysis

Each key secondary endpoint listed in Section 9.1 will be assessed based on MMRM. The change from baseline score for these scales at each scheduled postbaseline visit (according to the SoA) during the treatment period will be the dependent variable. The model for the fixed effects will include the following terms: baseline score, treatment, visit, treatment-by-visit interaction, baseline-by-visit interaction, *APOE* ε4 status, baseline CDR-GS, geographical regions and age at baseline. Visit will be considered a categorical variable. The null hypothesis is that the contrast between the remternetug group versus placebo at each visit equals 0. An unstructured covariance matrix will be used to model the within-subject variance-covariance errors. If the unstructured covariance structure matrix results in a lack of convergence, the following tests will be used in sequence:

1. (first) heterogeneous Toeplitz covariance structure
2. heterogeneous autoregressive covariance structure
3. heterogeneous compound symmetry covariance structure
4. (last) compound symmetry covariance structure.

The Kenward-Roger approximation will be used to estimate the denominator degrees of freedom. Due to the variable length of observation for each participant and the accumulation of events, the primary time point for treatment comparison will be specified in the SAP. The treatment group contrast in least-squares mean progression and its associated p-value and 95% CI will be calculated for the treatment comparison of remternetug versus placebo using the MMRM model specified above.

In addition to MMRM analysis described above, the Bayesian disease progression model, quadratic mixed effects models, and natural cubic spline models (Chambers and Hastie 1992) may be provided as sensitivity analyses for each key secondary endpoint. Additional models or analyses may be considered if deemed appropriate. The SAP will describe the planned analyses in greater detail.

9.3.4. Exploratory Endpoints Analysis

Other exploratory analyses, including but not limited to analyses of biomarkers, may be conducted if deemed appropriate and will be described in the SAP.

9.3.5. Safety Analyses

All safety analyses will be made on the safety population. Additional details will be provided in the SAP. Suicide-related thoughts and behaviors will be summarized based on responses to the C-SSRS consistent with the C-SSRS Scoring and Data Analysis Guide (C-SSRS WWW).

9.3.6. Other Analyses

9.3.6.1. Pharmacokinetic Analyses

To facilitate this modeling, and to ensure that exposure estimates from this study are available at the end of the trial, it is intended that the PK data will be locked after all participants complete 52 weeks of treatment, to allow PK modeling to begin before the end of the trial. No safety or efficacy data will be included in the 52-week PK lock. An Early PK Lock Plan will be developed and implemented prior to this lock, which will specify the safeguards to be taken to ensure the integrity of the study. It is intended that the results of the PK analysis will be provided in a separate report.

9.3.6.2. Evaluation of Immunogenicity

The frequency and percentage of participants with preexisting ADA and with TE ADA to remternetug may be tabulated.

TE ADA are defined as those with

- a titer 2-fold (1 dilution) greater than the minimum required dilution if no ADA were detected at baseline (treatment-induced ADA), or
- those with a 4-fold (2 dilutions) increase in titer compared to baseline if ADA were detected at baseline (treatment-boosted ADA).

The frequency of neutralizing antibodies may also be tabulated in TE ADA positive participants.

The relationship between the presence of antibodies and the PK parameters and PD response including safety and efficacy to remternetug may be assessed.

9.4. Interim Analyses

Interim analyses may be conducted for Study LAKI. Interims potentially executed may include, but are not limited to, safety, early efficacy, futility, sample size (based on number of events) or study duration re-estimation. If interim analyses of early efficacy are conducted, the protocol will be amended prior to the conduct of the interim analyses.

An independent external DMC will review the interim data. Only the DMC is authorized to evaluate results from unblinded interim efficacy and safety analyses and to recommend any modifications to the study, such as stopping the study. Study sites will receive information about interim results only if they need to know for the safety of trial participants or for changes to the protocol. For interims that stop the trial early, participants will proceed to their early discontinuation visit.

Unblinding details are specified in the unblinding plan section of the SAP or a separate unblinding plan document.

The SAP will describe the planned interim analyses in greater detail.

Operational details and a quantitative framework to provide information for decisions will be documented in the DMC charter. The timing of dissemination of data summaries based on interim analyses is addressed in Appendix 1, Section 10.1.6.

9.5. Sample Size Determination

Up to approximately 1400 participants will be enrolled in the trial.

The sample size was chosen to achieve approximately 350 clinical progression events within a trial duration of approximately 5 years after the first patient has been randomized. Assuming a hazard ratio of 0.725, a total of 350 events leads to approximately 85% power to show superiority of remternetug over placebo at a 1-sided 0.025 alpha level. The hazard ratio assumption was based on the treatment effect observed with other amyloid-targeting therapies in the early symptomatic AD population (van Dyck et al. 2023; Sims et al. 2023).

The additional assumptions used to calculate the sample size were an annualized placebo event rate of 8% to 10% (Sperling et al. 2023), and an annualized participant dropout rate of approximately 10%.

10. Supporting Documentation and Operational Considerations

10.1. Appendix 1: Regulatory, Ethical, and Study Oversight Considerations

10.1.1. Regulatory and Ethical Considerations

This study will be conducted in accordance with the protocol and with the following:

- consensus ethical principles derived from international guidelines, including the Declaration of Helsinki and Council for International Organizations of Medical Sciences International Ethical Guidelines
- applicable ICH GCP Guidelines
- ISO 14155, and
- applicable laws and regulations.

The protocol, protocol amendments, ICF, IB, and other relevant documents (for example, advertisements) must be submitted to an IRB/IEC by the investigator and reviewed and approved by the IRB/IEC before the study is initiated.

Any amendments to the protocol will require IRB/IEC approval before implementation of changes made to the study design, except for changes necessary to eliminate an immediate hazard to study participants.

Protocols and any substantial amendments to the protocol will require health authority approval prior to initiation except for changes necessary to eliminate an immediate hazard to study participants.

The investigator will be responsible for the following

- providing written summaries of the status of the study to the IRB/IEC annually or more frequently in accordance with the requirements, policies, and procedures established by the IRB/IEC
- notifying the IRB/IEC of SAEs or other significant safety findings as required by IRB/IEC procedures
- providing oversight of study conduct for participants under their responsibility and adherence to requirements of 21 CFR, ICH guidelines, the IRB/IEC, European regulation 536/2014 for clinical studies (if applicable), and all other applicable local regulations, and
- reporting to the sponsor or designee significant issues related to participant safety, participant rights, or data integrity.

Investigator sites are compensated for participation in the study as detailed in the CTA.

10.1.2. Financial Disclosure

Investigators and sub-investigators will provide the sponsor with sufficient, accurate financial information as requested to allow the sponsor to submit complete and accurate financial

certification or disclosure statements to the appropriate regulatory authorities. Investigators are responsible for providing information on financial interests during the course of the study and for 1 year after completion of the study.

10.1.3. Informed Consent Process

The investigator or the investigator's representative will explain the nature of the study, including the risks and benefits, to the potential participant or the potential participant's legally authorized representative, and the potential participant's study partner, and answer all questions regarding the study.

Potential participants and their study partners must be informed that their participation is voluntary. Participants or their legally authorized representative and their study partners will be required to sign a statement of informed consent that meets the requirements of 21 CFR 50, local regulations, ICH guidelines, privacy and data protection requirements, where applicable, and the IRB/IEC or study center.

The medical record must include a statement that written informed consent was obtained before the participant and their study partner were entered in the study, and the date the written consent was obtained. The authorized person obtaining the informed consent must also sign the ICF.

Revised consents must be appropriately obtained using the correct approved ICFs for applicable study participants in accordance with sponsor and ERB consenting guidance.

eConsent may be used in this study.

A copy of the ICFs must be provided to the participant or to the participant's legally authorized representative, and their study partner, and is kept on file.

10.1.4. Data Protection

Participants will be assigned a unique identifier by the sponsor to protect the participant's personal data. Any participant information, such as records, datasets or tissue samples that are transferred to the sponsor will contain the identifier only. Participant names or any information which would make the participant identifiable will not be transferred.

The participant must be informed that the participant's personal study-related data will be used by the sponsor in accordance with local data protection law. The level of disclosure must also be explained to the participant who will be required to give consent for their data to be used as described in the informed consent. This is done by the site personnel through the informed consent process.

The participant must be informed through the informed consent by the site personnel that their medical records may be examined by Clinical Quality Assurance auditors or other authorized personnel appointed by the sponsor, by appropriate IRB/IEC members, and by inspectors from regulatory authorities.

The sponsor has processes in place to ensure information security, data integrity, and data protection. These processes address management of data transfer, and prevention and management of unauthorized access, disclosure, dissemination, alteration or loss of information or personal data. These processes include appropriate contingency plans for appropriate and timely response in the event of a data security breach.

The transfer of personal data is subject to appropriate safeguards through contractual agreements and processes. The sponsor's processes are compliant with local privacy laws and relevant legislations including the General Data Protection Regulation.

10.1.5. Committees Structure

Data Monitoring Committee (DMC)

A DMC consisting of members external to Lilly will be established. The purpose of the DMC is to conduct periodic monitoring of the clinical study data. The DMC will consist of a minimum of 3 members, including a physician with expertise in AD and a statistician.

No member of the DMC will influence study sites. A SAC will prepare and provide unblinded data to the DMC. The SAC members will not be Lilly employees but will come from TPOs designated by Lilly. The SAC members will have no contact with sites and no privileges to influence changes to the ongoing studies. The timing and frequency of the periodic clinical study data review by the DMC will be detailed in a DMC charter.

The DMC is authorized to evaluate unblinded interim and safety analyses. In addition, the DMC may request key efficacy data to put safety observations into context and to assess a reasonable benefit/risk profile for ongoing participants in the study. The DMC will make recommendation to the sponsor, who may order the immediate implementation of the DMC recommendation, or may convene an internal review committee, which is independent from the study team, to review the recommendation.

Study sites will receive information about interim results only if it is required for the safety of their participants.

Committee for adjudication of clinical events

A committee for the adjudication of the etiology of clinical progression events will also be established. The committee will adjudicate whether clinical progression events are consistent with worsening due to AD or consistent with other etiology. Details will be included in the adjudication committee charter.

10.1.6. Dissemination of Clinical Study Data

Reports

The sponsor will disclose a summary of study information, including tabular study results, on publicly available websites where required by local law or regulation.

The summary of results will be posted within the time frame specified by local law or regulation. If the study remains ongoing in some countries and a statistical analysis of an incomplete dataset would result in analyses lacking scientific rigor (for example, underpowered) or compromise the integrity of the overall analyses (for example, trial not yet unblinded), the summary of results will be submitted within 1 year after the end of the study globally or as soon as available, whichever is earlier.

Data

The sponsor provides access to all individual participant data collected during the trial, after anonymization, with the exception of pharmacokinetic or genetic data.

Data are available to request 6 months after the indication studied has been approved in the US and EU and after primary publication acceptance, whichever is later. No expiration date of data requests is currently set once data are made available.

Access is provided after a proposal has been approved by an independent review committee identified for this purpose and after receipt of a signed data-sharing agreement.

Data and documents, including the study protocol, SAP, CSR, and blank or annotated CRFs, will be provided in a secure data-sharing environment for up to 2 years per proposal.

For details on submitting a request, see the instructions provided at www.vivli.org.

10.1.7. Data Quality Assurance

All participant data relating to the study will be recorded on printed or electronic CRFs unless transmitted to the sponsor or designee electronically (for example, laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by physically or electronically signing the CRF.

The investigator must maintain accurate documentation (source data) that supports the information entered in the CRF. This includes laboratory tests, medical records, and clinical notes.

The investigator must review and confirm that data entries are accurate and complete throughout the duration of the study, by physically or electronically signing the CRF, as instructed by the sponsor. All completed CRFs must be signed prior to archival.

The investigator must permit study-related monitoring, audits, IRB/IEC review, and regulatory agency inspections and provide direct or remote access to source documents.

QTLs will be predefined to identify systematic issues that can impact participant safety and/or reliability of study results. These predefined parameters will be monitored during the study and important excursions from the QTLs and remedial actions taken will be summarized in the CSR.

Monitoring details describing strategy, for example, risk-based initiatives in operations and quality such as risk management and mitigation strategies and analytical risk-based monitoring, methods, responsibilities and requirements, including handling of noncompliance issues and monitoring techniques are provided in the Monitoring Plan.

The sponsor or designee is responsible for the data management of this study including quality checking of the data.

The sponsor assumes accountability for actions delegated to other individuals, for example, contract research organizations.

The sponsor or designee will perform monitoring to confirm that data transcribed into the CRF by authorized site personnel are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is being conducted in accordance with the currently approved protocol and any other study agreements, ICH GCP, and all applicable regulatory requirements.

Records and documents, including signed ICFs, pertaining to the conduct of this study must be retained by the investigator for the time period outlined in the CTA unless local regulations or

institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the sponsor. No records may be transferred to another location or party without written notification to the sponsor.

In addition, the sponsor or its representatives will periodically check a sample of the participant data recorded against source documents at the study site. The study may be audited by the sponsor or its representatives, or regulatory agencies at any time. Investigators will be given notice before an audit occurs.

Data capture system

The investigator is responsible for ensuring the accuracy, completeness, legibility, and timeliness of the data reported to the sponsor.

An EDC system will be used in this study for the collection of CRF data. The investigator maintains a separate source for the data entered by the investigator or designee into the sponsor-provided EDC system. The investigator is responsible for the identification of any data to be considered source and for the confirmation that data reported are accurate and complete by signing the CRF.

Additionally, eCOA data (participant-focused outcome instrument) will be directly recorded by the participant, study partner, and/or authorized site personnel, into an instrument (for example, handheld smart phone or tablet). The eCOA data will serve as the source documentation and the investigator does not maintain a separate, written or electronic record of these data.

Additionally, dosing diary data will be recorded directly by the participant or study partner via a paper source document. Dosing diary data recorded via a paper source document will be transcribed by the authorized study personnel into the electronic data capture system and will serve as the source documentation. Furthermore, SC study drug accountability will be assessed by CRF and dosing diary review.

Data collected via the sponsor-provided data capture systems will be stored at third parties. The investigator will have continuous access to the data during the study and until decommissioning of the data capture systems. Prior to decommissioning, the investigator will receive or access an archival copy of pertinent data for retention.

Data managed by a central vendor, such as laboratory test data, will be stored electronically in the central vendor's database system and reports or electronic transfers will be provided to the investigator for review and retention. Data will subsequently be transferred from the central vendor to the sponsor data warehouse.

Data from complaint forms submitted to the sponsor will be encoded and stored in the global PC management system.

10.1.8. Source Documents

Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents are filed at the investigator's site.

Data reported on or entered in the CRF and are transcribed from source documents must be consistent with the source documents or the discrepancies must be explained. The investigator

may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

Definition of what constitutes source data can be found in the site confirmation of source data.

10.1.9. Study and Site Start and Closure

Study start

The study start date is the date on which the clinical study will be open for recruitment of participants.

First act of recruitment

The first act of recruitment is the opening of the first site.

Study or site termination

The sponsor or sponsor's designee reserves the right to close the study site or terminate the study at any time for any reason at the sole discretion of the sponsor. Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed.

The investigator may initiate study-site closure at any time, provided there is reasonable cause and sufficient notice is given in advance of the intended termination.

Reasons for the early closure of a study site by the sponsor or investigator may include but are not limited to

- for study termination due to discontinuation of further study intervention development
- for site termination due to
  - failure of the investigator to comply with the protocol, the requirements of the IRB/IEC or local health authorities, the sponsor's procedures, or GCP guidelines
  - inadequate recruitment, evaluated after a reasonable amount of time of participants by the investigator, or
  - total number of participants included earlier than expected.

If the study is prematurely terminated or suspended, the sponsor shall promptly inform the investigators, the IECs/IRBs, the regulatory authorities, and any contract research organizations used in the study of the reason for termination or suspension, as specified by the applicable regulatory requirements. The investigator shall promptly inform the participant and should assure appropriate participant therapy and/or follow-up.

10.1.10. Publication Policy

In accordance with the sponsor's publication policy, the results of this study will be submitted for publication by a peer-reviewed journal.

10.1.11. Investigator Information

Researchers with appropriate education, training, and experience, as determined by the sponsor, will participate as investigators in this clinical trial.

10.1.12. Sample Retention

Samples will be retained at a facility selected by the sponsor or its designee.

Sample retention enables the use of new technologies, response to regulatory questions, and investigation of variable response that may not be observed until later in the development of the study intervention or after the study intervention becomes commercially available.

This table lists the maximum retention period for sample types.

Sample Type	Custodian	Maximum Retention Period after the Last Participant Visit
Pharmacokinetics	Sponsor or designee	1 year
Biomarkers (including P-tau and exploratory)	Sponsor or designee	Up to 7 years
Immunogenicity	Sponsor or designee	Up to 15 years
Genetics	Sponsor or designee	Up to 7 years

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10.2. Appendix 2: Clinical Laboratory Tests

Use of central or local laboratories

The tests detailed in the following tables will be performed by the Lilly-designated laboratory or by the local laboratory as specified in the tables.

In circumstances where the sponsor approves local laboratory testing in lieu of central laboratory testing (in the table below), the local laboratory must be qualified in accordance with applicable local regulations.

Additional laboratory testing

Additional tests may be performed at any time during the study as determined necessary by the investigator or required by local regulations.

Investigator responsibilities

Investigators must document their review of the laboratory safety results.

Provision of laboratory test results

Laboratory test results that could unblind the study will not be reported to investigative sites or other blinded personnel.

10.2.1. Clinical Laboratory Tests

	Comments
Hematology	Assayed by Lilly-designated laboratory
Hemoglobin	
Hematocrit	
Erythrocyte count (RBCs –- red blood cells)	
Mean cell volume	
Mean cell hemoglobin	
Mean cell hemoglobin concentration	
Leukocytes (WBCs –- white blood cells)	
Differential	
Neutrophils, segmented	
Lymphocytes	
Monocytes	
Eosinophils	
Basophils	
Platelets	
Cell morphology (RBCs and WBCs)	

	Comments
Clinical Chemistry	Assayed by Lilly-designated laboratory
Sodium	
Potassium	
Chloride	
Bicarbonate	
Total bilirubin	
Direct bilirubin	
Alkaline phosphatase (ALP)	
Alanine aminotransferase (ALT)	
Aspartate aminotransferase (AST)	
Gamma-glutamyl transferase (GGT)	
Blood urea nitrogen (BUN)	
Creatinine	
Creatine kinase (CK)	
Uric acid	
Albumin	
Calcium	
Glucose	
Cholesterol	

	Comments
Urinalysis	Assayed by Lilly-designated laboratory
Specific gravity	
pH	
Protein	
Glucose	
Ketones	
Blood	
Urine leukocyte esterase	

	Comments
Pharmacokinetic Samples – serum remternetug concentration	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites.

	Comments
Biomarkers	Assayed by Lilly-designated laboratory.
Plasma phosphorylated tau (P-tau)	Investigator will be provided information about participant eligibility. Samples can be stored.

	Comments
Genetics Samples	Assayed by Lilly-designated laboratory.
Genetics sample (stored)	Results will not be provided to the investigative sites.
APOE	See Appendix 7, Section 10.7.

	Comments
Exploratory Biomarker Storage Samples	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites.
Serum	
Plasma	
Whole blood for RNA	

	Comments
Immunogenicity (Anti-Drug Antibodies) Samples	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites.
Anti-remternetug antibodies	
Anti-remternetug antibodies neutralization	

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10.2.2. Laboratory Samples to be Obtained at the Time of a Systemic Hypersensitivity Event

Purpose of collecting samples after a systemic hypersensitivity event

The samples listed in this appendix are not collected for acute study participant management. The sponsor will use the laboratory tests results from these samples to characterize hypersensitivity events across the clinical development program.

When to collect samples after a systemic hypersensitivity event occurs

Collect the samples listed below if a systemic hypersensitivity event is suspected. The timing should be as designated in the table, assuming the participant has been stabilized.

Obtain follow-up predose samples at the next regularly scheduled laboratory sample collection (ideally prior to the next dose after the event) to assess postevent return-to-baseline values.

Timing	Laboratory Test [a]
Collect from 30 min to 4 hr after the start of the event. ● Note: The optimal collection time is from 1 to 2 hr after the start of event.	total tryptase complements (C3, C3a, and C5a) cytokine panel (IL-6, IL-1β, IL-10 or any cytokine panel that includes these 3 cytokines)
Collect only if not already collected on the same day as the event. ● Note: If collecting, collect up to 12 hr after the start of the event.	remternetug anti-drug antibodies (ADA) remternetug concentration

a All samples for hypersensitivity testing will be assayed by Lilly-designated laboratory. Results will not be provided to the study site. If samples are not collected or are collected outside the specified time period, this will not be considered a protocol deviation.

Abbreviation: IL = interleukin.

What information to record

Record the date and time when the samples are collected.

Allowed additional testing for participant management

The investigator may perform additional tests locally, if clinically indicated, for acute study participant management.

10.3. Appendix 3: Adverse Events and Serious Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-up, and Reporting

The definitions and procedures detailed in this appendix are in accordance with ISO 14155.

Both the investigator and the sponsor will comply with all local medical device reporting requirements, if applicable.

The detection and documentation procedures described in this protocol apply to all sponsor medical devices provided for use in the study, if any are listed in this protocol.

10.3.1. Definition of AE

An AE is any untoward medical occurrence in a participant administered a pharmaceutical product and which does not necessarily have a causal relationship with the study intervention. An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of a medicinal (investigational) product, or investigational combination product, whether or not related to the medicinal (investigational) product or investigational combination product.

An AE is also any untoward medical occurrence, unintended disease or injury, or untoward clinical signs (including abnormal laboratory finding) in study participants, users, or other persons, whether or not related to the investigational device.

An ADE is defined as an AE related to the use of an investigational medical device. This definition includes any AEs resulting from insufficient or inadequate instructions for use, deployment, implantation, installation, or operation; any malfunction of the investigational medical device as well as any event resulting from use error or from intentional misuse of the investigational medical device.

Events meeting the AE definition

- Any abnormal laboratory test results (hematology, clinical chemistry, or urinalysis) or other safety assessments, for example, ECG, radiological scans, and vital signs measurements, including those that worsen from baseline, considered clinically significant in the medical and scientific judgment of the investigator, that is, not related to progression of underlying disease.
- Exacerbation of a chronic or intermittent preexisting condition including either an increase in frequency and/or intensity of the condition.
- New condition detected or diagnosed after study intervention administration even though it may have been present before the start of the study.
- Signs, symptoms, or the clinical sequelae of a suspected drug-drug interaction.
- Medication error, misuse, or abuse of IMP, including signs, symptoms, or clinical sequelae.

- Lack of efficacy or failure of expected pharmacological action per se will not be reported as an AE or SAE. Such instances will be captured in the efficacy assessments. However, the signs, symptoms, and/or clinical sequelae resulting from lack of efficacy will be reported as AE or SAE if they fulfill the definition of an AE or SAE.

Events NOT meeting the AE definition

In line with clinical practice for this disease state, and based on the defined criteria for AEs and related clarifications (ICH 1994; FDA 2009; EMA 2017; CFR 2024), the following are not considered AEs:

- Any clinically significant abnormal laboratory findings or other abnormal safety assessments that are associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.
- The disease or disorder being studied or expected progression, signs, or symptoms of the disease or disorder being studied, unless more severe than expected for the participant's condition.
- Situations in which an untoward medical occurrence did not occur (social and/or convenience admission to a hospital).
- Anticipated day-to-day fluctuations of preexisting diseases or conditions present or detected at the start of the study that do not worsen.

10.3.2. Definition of SAE

An SAE is defined as any untoward medical occurrence that, at any dose, meets 1 or more of the criteria listed:

- Results in death
- Is life-threatening
  - The term *life-threatening* in the definition of *serious* refers to an event in which the participant was at risk of death at the time of the event. It does not refer to an event, which hypothetically might have caused death, if it were more severe.
- Requires inpatient hospitalization or prolongation of existing hospitalization
  - In general, hospitalization signifies that the participant has been admitted to hospital or emergency ward (usually involving at least an overnight stay) for observation and/or treatment that would not have been appropriate in the physician's office or outpatient setting. Complications that occur during hospitalization are AEs. If a complication prolongs hospitalization or fulfills any other serious criteria, the event is serious. When in doubt as to whether hospitalization occurred or was necessary, the AE should be considered serious.
  - Hospitalization for elective treatment of a preexisting condition that did not worsen from baseline is not considered an AE.

- Results in persistent disability/incapacity
  - The term disability means a substantial disruption of a person's ability to conduct normal life functions.
  - This definition is not intended to include experiences of relatively minor medical significance, such as uncomplicated headache, nausea, vomiting, diarrhea, influenza, and accidental trauma, for example, sprained ankle, which may interfere with or prevent everyday life functions but do not constitute a substantial disruption.
- Is a congenital anomaly/birth defect
  - Abnormal pregnancy outcomes, for example, spontaneous abortion, fetal death, stillbirth, congenital anomalies, and ectopic pregnancy, are considered SAEs.
- Other situations:
  - Medical or scientific judgment should be exercised by the investigator in deciding whether SAE reporting is appropriate in other situations such as important medical events that may not be immediately life-threatening or result in death or hospitalization but may jeopardize the participant or may require medical or surgical intervention to prevent one of the other outcomes listed in the above definition. These events should usually be considered serious.

    Examples of such events include invasive or malignant cancers, intensive treatment in an emergency room or at home for allergic bronchospasm, blood dyscrasias or convulsions that do not result in hospitalization, or development of drug dependency or drug abuse.
- Resulted in medical or surgical intervention to prevent life-threatening illness or injury or permanent impairment to a body structure or a body function.
- A serious ADE is defined as an ADE that has resulted in any of the consequences characteristic of an SAE.
- An unanticipated ADE is a serious adverse effect on health or safety or any life-threatening problem or death caused by or associated with a device, if that effect, problem, or death was not previously identified in nature, severity, or degree of incidence in the investigational plan or application (including a supplementary plan or application), or any other unanticipated serious problem associated with a device that relates to the rights, safety, or welfare of the participant. An unanticipated serious ADE is equivalent to an unanticipated ADE.

10.3.3. Definition of Product Complaints

Product complaint

A PC is any written, electronic, or oral communication that alleges deficiencies related to the identity, quality, durability, reliability, safety, effectiveness or performance of a study

intervention. When the ability to use the study intervention safely is impacted, the following are also PCs:

- deficiencies in labeling information, and
- use errors for device or drug-device combination products due to ergonomic design elements of the product.

PCs related to study interventions used in clinical trials are collected to ensure the safety of participants, monitor quality, and to facilitate process and product improvements.

If the participant identifies a PC or a problem with the study intervention, investigators will instruct participants to contact the site as soon as possible so that the situation can be assessed.

An event may meet the definition of both a PC and an AE/SAE. In such cases, it should be reported as both a PC and as an AE/SAE.

Device deficiencies are PCs.

10.3.4. Recording and Follow-Up of AE and/or SAE and Product Complaints

AE, SAE, and product complaint recording

When an AE/SAE/PC occurs, it is the responsibility of the investigator to review all documentation, for example, hospital progress notes, laboratory reports, and diagnostics reports, related to the event.

The investigator will then record all relevant AE/SAE/PC information in the participant's medical records, in accordance with the investigator's normal clinical practice. AE/SAE information is reported on the appropriate CRF page and product complaint information is reported on the PC Form.

Note: An event may meet the definition of both a PC and an AE/SAE. In such cases, it should be reported as both a PC and as an AE/SAE.

It is **not** acceptable for the investigator to send photocopies of the participant's medical records to the sponsor or designee in lieu of completion of the CRF page for AE/SAE and the PC Form for product complaints.

There may be instances when copies of medical records for certain cases are requested by the sponsor or designee. In this case, all participant identifiers, with the exception of the participant number, will be redacted on the copies of the medical records before submission to the sponsor or designee.

The investigator will attempt to establish a diagnosis of the event based on signs, symptoms, and/or other clinical information. Whenever possible, the diagnosis (not the individual signs/symptoms) will be documented as the AE/SAE.

Assessment of intensity

The investigator will make an assessment of intensity for each AE and SAE reported during the study and assign it to one of the following categories:

- Mild — A type of AE that is usually transient and may require only minimal treatment or therapeutic intervention. The event does not generally interfere with usual activities of daily living.

- Moderate — A type of AE that is usually alleviated with additional specific therapeutic intervention. The event interferes with usual activities of daily living, causing discomfort but poses no significant or permanent risk of harm to the research participant.
- Severe — A type of AE that interrupts usual activities of daily living, or significantly affects clinical status, or may require intensive therapeutic intervention. An AE that is assessed as severe should not be confused with an SAE. Severe is a category utilized for rating the intensity of an event, and both AEs and SAEs can be assessed as severe.

  An event is defined as "serious" when it meets at least 1 of the predefined outcomes as described in the definition of an SAE, NOT when it is rated as severe.

Assessment of causality

The investigator is obligated to assess the relationship between study intervention and each occurrence of each AE/SAE. The investigator will use clinical judgment to determine the relationship.

A "reasonable possibility" of a relationship conveys that there are facts, evidence, and/or arguments to suggest a causal relationship, rather than a relationship cannot be ruled out.

Alternative causes, such as underlying diseases, concomitant therapy, and other risk factors, as well as the temporal relationship of the event to study intervention administration will be considered and investigated.

The investigator will also consult the IB in their assessment.

The investigator **must** review and provide an assessment of causality for each AE/SAE and document this in the medical notes.

There may be situations in which an SAE has occurred and the investigator has minimal information to include in the initial report to the sponsor or designee. However, it is very important that the investigator always make an assessment of causality for every event before the initial transmission of the SAE data to the sponsor or designee.

The investigator may change their opinion of causality in light of follow-up information and send an SAE follow-up report with the updated causality assessment.

The causality assessment is one of the criteria used when determining regulatory reporting requirements.

Follow-up of AEs and SAEs

The investigator is obligated to perform or arrange for the conduct of supplemental measurements and/or evaluations as medically indicated or as requested by the sponsor or designee to elucidate the nature and/or causality of the AE or SAE as fully as possible. This may include additional laboratory tests or investigations, histopathological examinations, or consultation with other healthcare professionals.

If a participant dies during participation in the study or during a recognized follow-up period, the investigator will provide the sponsor or designee with a copy of any postmortem findings including histopathology.

10.3.5. Reporting of SAEs

SAE reporting via an electronic data collection tool

The primary mechanism for reporting an SAE will be the electronic data collection tool.

If the electronic system is unavailable, then the site will use the SAE paper form (see next section) to report the event within 24 hours.

The site will enter the SAE data into the electronic system as soon as it becomes available.

After the study is completed at a given site, the electronic data collection tool will be taken off-line to prevent the entry of new data or changes to existing data.

If a site receives a report of a new SAE from a study participant or receives updated data on a previously reported SAE after the electronic data collection tool has been taken off-line, then the site can report this information on an SAE paper form (see next section) or to the sponsor or designee by telephone.

Contacts for SAE reporting can be found in Global Patient Safety Clinical Trial Serious Adverse Event (SAE) Transmission Cover Sheet and Form.

SAE reporting via paper form

Facsimile transmission of the SAE paper form is the preferred method to transmit this information to the sponsor or designee.

Initial notification via telephone does not replace the need for the investigator to complete and sign the SAE CRF pages within the designated reporting time frames.

Contacts for SAE reporting can be found in Global Patient Safety Clinical Trial Serious Adverse Event (SAE) Transmission Cover Sheet and Form.

10.3.6. Regulatory Reporting Requirements

SAE regulatory reporting

Prompt notification by the investigator to the sponsor of an SAE is essential so that legal obligations and ethical responsibilities toward the safety of participants and the safety of a study intervention under clinical investigation are met.

The sponsor has a legal responsibility to notify both the local regulatory authority and other regulatory agencies about the safety of a study intervention under clinical investigation. The sponsor will evaluate the reported SAEs, including confirmation of relatedness and assessment of expectedness. The sponsor will comply with country-specific regulatory requirements relating to safety reporting to the regulatory authority, IRB/IEC, and investigators.

An investigator who receives an investigator safety report describing an SAE or other specific safety information, for example, summary or listing of SAEs, from the sponsor will review and then file it along with the IB and will notify the IRB/IEC, if appropriate according to local requirements.

10.4. Appendix 4: Contraceptive and Barrier Guidance

10.4.1. Definitions

Word/Phrase	Definition
Female	Female sex based on external genitalia and/or genetic or medical information. In addition, if these individuals are of reproductive potential, they are potentially capable of gestating a fetus, and thus are capable of exposing an egg, embryo, or fetus to study drug or drug effects.
Male	Male sex based on external genitalia and/or genetic or medical information. In addition, if these individuals are of reproductive potential, they are not capable of gestating a fetus, but are capable of exposing a fetus to study drug or drug effects via their semen. Males are considered to be not of reproductive potential if they have had orchiectomy (bilateral orchidectomy) with or without penectomy, confirmed by operative note.
Individuals of childbearing potential (IOCBP) [a]	Adult females are considered IOCBP unless they are INOCBP. Note: Adolescent or adult females who are receiving hormone therapy as part of gender transition are considered IOCBP unless they meet the conditions outlined below for INOCBP.
Individuals not of childbearing potential (INOCBP) [b]	Females are considered INOCBP if they are not capable of producing ova or embryo, and/or are not capable of potentially gestating a fetus. Such individuals include those who ● have a congenital anomaly such as Müllerian agenesis, resulting in confirmed infertility ● are infertile due to surgical sterilization, or ● are menopausal. Acceptable surgical sterilization methods are hysterectomy, bilateral salpingo-oophorectomy, bilateral salpingectomy, or bilateral oophorectomy.
Menopausal state [c]	The menopausal state is defined as an individual: ● at any age at least 6 weeks postsurgical bilateral oophorectomy with or without hysterectomy, confirmed by operative note; or ● aged at least 40 years and up to 55 years with an intact uterus, not on hormone therapy[c], who has had cessation of menses for at least 12 consecutive months without an alternative medical cause, AND with a follicle-stimulating hormone ≥40 mIU/mL; or ● 55 years or older not on hormone therapy, who has had at least 12 months of spontaneous amenorrhea, or ● aged at least 55 years with a diagnosis of menopause prior to starting hormone replacement therapy.

a IOCBP is inclusive of the concept of women of childbearing potential (WOCBP or WCBP), a term often used in literature and regulatory guidance documents.

b INOCBP is inclusive of the concept of women not of childbearing potential (WNOBCP).

c The individual **should not** be taking medications during amenorrhea such as oral contraceptives, hormone replacement therapy (HRT), gonadotropin-releasing hormone, anti-estrogens, selective estrogen receptor modulators, or chemotherapy that could induce transient amenorrhea. Individuals on HRT and those whose menopausal status cannot be confirmed will be required to comply with the protocol contraception requirements if they wish to continue HRT during the study. Otherwise they must discontinue HRT to allow confirmation of menopausal status before study enrollment.

10.4.2. Contraception Guidance

This table provides a summary of study eligibility according to individual's childbearing potential and, if applicable, any contraception requirements for the duration of this study.

Reproductive Potential	Study Eligibility	Contraception Requirements
Males	may participate	Contraception is not required except as necessary to comply with specific local government requirements for clinical trials.
Women not of childbearing potential (INOCBP)	may participate	Not applicable
Women of childbearing potential (IOCBP)	may **not** participate	Not applicable

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10.5. Appendix 5: Liver Safety: Suggested Actions and Follow-up Assessments

Hepatic evaluation testing

See Section 8.2.6 for guidance on appropriate test selection.

The Lilly-designated central laboratory should complete the analysis of all selected testing except for testing listed in the investigator-designated local laboratory table. The central laboratory will report results if a validated test or calculation is available.

In circumstances where required, in accordance with local regulations, local laboratory testing may be performed in lieu of Lilly-designated central laboratory testing (in the table below).

Local testing may be performed *in addition to central testing* when necessary for immediate participant management.

The local laboratory must be qualified in accordance with applicable local regulations. Note that availability of testing may vary based on local regulatory requirements or regional availability of test. If testing is not available based on local requirements, this will not be considered a protocol deviation, and the sponsor's designated medical monitor should be consulted.

Tests available from by Lilly-designated central laboratory	
Hepatic Hematology Panel	**Hepatitis A virus (HAV) testing:**
Hemoglobin	HAV total antibody
Hematocrit	HAV IgM antibody
Erythrocytes (RBCs - red blood cells)	**Hepatitis B virus (HBV) testing:**
Leukocytes (WBCs - white blood cells)	Hepatitis B surface antigen (HBsAg)
Differential:	Hepatitis B surface antibody (anti-HBs)
Neutrophils	Hepatitis B core total antibody (anti-HBc)
Lymphocytes	Hepatitis B core IgM antibody
Monocytes	HBV DNA
Basophils	**Hepatitis C virus (HCV) testing:**
Eosinophils	HCV antibody (total or IgG)
Platelets	HCV RNA
Cell morphology (RBC and WBC)	
Hepatic Clinical Chemistry Panel	
Total bilirubin	
Direct bilirubin	
Alkaline phosphatase (ALP)	**Hepatitis E virus (HEV) testing:**
Alanine aminotransferase (ALT)	HEV IgG antibody
Aspartate aminotransferase (AST)	HEV IgM antibody
Gamma-glutamyl transferase (GGT)	HEV RNA
Creatine kinase (CK)	**Anti-nuclear antibody (ANA)**
Hepatic Coagulation Panel	**Anti-smooth muscle antibody (ASMA) or anti-actin antibody**
Prothrombin time, INR (PT-INR)	**Immunoglobulin IgA (quantitative)**
Urine Chemistry	**Immunoglobulin IgG (quantitative)**
Drug screen	**Immunoglobulin IgM (quantitative)**
Haptoglobin	

Tests to be performed by investigator-designated laboratory, as needed	
Acetaminophen	**Cytomegalovirus (CMV) testing:**
Acetaminophen protein adducts	CMV antibody
Alkaline phosphatase isoenzymes	CMV DNA
Ceruloplasmin	**Herpes simplex virus (HSV) testing:**
Copper	HSV (Type 1 and 2) antibody
Ethyl alcohol (EtOH)	HSV (Type 1 and 2) DNA
Phosphatidylethanol (pEth)	Liver kidney microsomal type 1 (LKM-1) antibody
	Microbiology Culture:
Ethyl glucuronide (EtG)	Blood
Epstein-Barr virus (EBV) testing:	Urine
EBV DNA	**Hepatitis D virus (HDV) testing:**
EBV antibody	HDV total antibody
Iron	HDV IgM antibody
Ferritin	HDV RNA
Total Iron Binding Capacity or Transferrin Saturation	

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10.6. Appendix 6: Region-Specific or Country-Specific Requirements

10.6.1. European Union

The study will be conducted in accordance with the Regulation (EU) No 536/2014.

Regulatory reporting requirements

The sponsor has processes for safety reports for identification, recording, and expedited reporting of SUSARs according to EU regulatory requirements. The sponsor will comply with applicable regulatory requirements relating to safety reporting to the regulatory authority, per European Union Clinical Trial Regulation 536/2014 submission of SUSARs to the EudraVigilance database.

Provisions for Changes in Study Conduct During Exceptional Circumstances

For the purposes of Appendix 9, Section 10.9, COVID-19 is not considered an ongoing pandemic.

10.6.2. Japan

Informed consent

For sites in Japan, the initial informed consent may be collected in person or remotely or virtually in accordance with the local regulations of the local institution.

If the informed consent form is revised, then written informed consent can be collected remotely or virtually in accordance with the local regulations of the local institution.

General statistical considerations

Japanese participants will be included in the primary analysis and all main study analyses in which the cognitive and functional assessment data are collected.

10.6.3. South Korea

Contraception Guidance

IOCBP are not eligible to enroll. However, if inadvertently enrolled, such participants must use effective contraception during the administration period and for 3 months after the administration period.

10.7. Appendix 7: Optional APOE Disclosure

All consented study participants will have an *APOE* sample collected at Visit 601, as shown in the SoA (Section 1.3). Refer to Section 8.6.1 for more details.

10.8. Appendix 8: Definition of Early Alzheimer's Disease

Stage 1: Patients with characteristic pathophysiological changes of AD but no evidence of clinical impact. These patients are truly asymptomatic with no subjective complaint, functional impairment, or detectable abnormalities on sensitive neuropsychological measures. The characteristic pathophysiological changes are typically demonstrated by assessment of various biomarker measures.
Stage 2: Patients with characteristic pathophysiological changes of AD and subtle detectable abnormalities on sensitive neuropsychological measures or subjective complaints of mild cognitive symptoms but no functional impairment. This may be considered a transitional stage in which slight cognitive symptoms first appear. The emergence of subtle functional impairment signals a transition to Stage 3.
Stage 3: Patients with characteristic pathophysiological changes of AD, generally more apparent detectable abnormalities on sensitive neuropsychological measures, and mild but detectable functional impairment. The functional impairment in this stage is not severe enough to warrant a diagnosis of overt dementia. This stage roughly corresponds with the syndrome of "mild cognitive impairment"; however, it is noted that the term "mild cognitive impairment" may also encompass patients in late Stage 2 or early Stage 4.

Source: FDA 2024.

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10.9. Appendix 9: Provisions for Changes in Study Conduct During Exceptional Circumstances

Implementation of this appendix

The changes to procedures described in this appendix are temporary measures intended to be used only during specific time periods as directed by the sponsor in partnership with the investigator.

In an exceptional circumstance, the sponsor's procedures ensure that relevant parties are kept informed of changes to the design and conduct of the study.

Exceptional circumstances

Exceptional circumstances are rare events that may cause disruptions to the conduct of the study. Examples include pandemics or natural disasters. These disruptions may limit the ability of the investigators, participants, or both to attend on-site visits or to conduct planned study procedures.

Implementing changes under exceptional circumstances

In an exceptional circumstance, after receiving the sponsor's written approval, sites may implement changes if permitted by local regulations.

After approval by local ERBs, regulatory bodies and any other relevant local authorities, implementation of these exceptional circumstance changes will not typically require additional notification to these groups, unless they have specific requirements in which notification is required (for example, upon implementation and suspension of changes). All approvals and notifications must be retained in the study records.

If the sponsor grants written approval for changes in study conduct, the sponsor will also provide additional written guidance, if needed.

Considerations for making a change

The prevailing consideration for making a change is ensuring the safety of study participants. Additional important considerations for making a change are compliance with Good Clinical Practice, enabling participants to continue safely in the study and maintaining the integrity of the study.

Informed consent

Additional consent from the participant will be obtained, if required, for:

- participation in remote visits not originally specified as such in the protocol, as defined in Section "Remote Visits"
- a change in the method of study intervention administration
- dispensation of additional study intervention during an extended treatment period
- alternate delivery of study intervention and ancillary supplies, and
- provision of their personal or medical information required prior to implementation of these activities.

Changes in study conduct during exceptional circumstances

Changes in study conduct not described in this appendix, or not consistent with applicable local regulations, are not allowed.

The following changes in study conduct will not be considered protocol deviations.

Remote visits

Types of remote visits in addition to planned remote activities

Mobile healthcare: Healthcare visits may be performed by a mobile healthcare provider at locations other than the study site when participants cannot travel to the site due to an exceptional circumstance if written approval is provided by the sponsor. Procedures performed at such mobile healthcare visits include, but are not limited to, informed consent, collection of blood or urine samples, physical assessments, administration of study intervention and/or dosing diaries, and collection of health information.

Other alternative locations: Procedures that may be done at an alternate location in very exceptional circumstances include, but are not limited to, MRIs, ECGs, collections of blood and urine samples.

Data capture

In source documents and the CRF, the study site should capture the visit method, with a specific explanation for any data missing because of missed in-person site visits.

Safety reporting

Regardless of the type of remote visits implemented, the protocol requirements regarding the reporting of AEs, SAEs, and PCs remain unchanged.

Return to on-site visits

Every effort should be made to enable participants to return to scheduled on-site visits as soon as reasonably possible, while ensuring the safety of both the participants and the site staff.

Local laboratory testing option

Local laboratory testing may be conducted in lieu of central laboratory testing for hematology, clinical chemistry, and urinalysis. Central laboratory testing must be retained for all other laboratory samples. The local laboratory must be qualified in accordance with applicable local regulations.

Study intervention and ancillary supplies (including participant diaries)

When a participant is unable to go to the site to receive study supplies during normal on-site visits, the site should work with the sponsor to determine appropriate actions. These actions may include

- asking the participant to go to the site and receive study supplies from site staff without completion of a full study visit
- asking the participant's designee to go to the site and receive study supplies on a participant's behalf
- arranging delivery of study supplies, and

- working with the sponsor to determine how study intervention that is typically administered on site will be administered to the participant; for example, at an alternate location.

These requirements must be met before action is taken:

- Alternate delivery of study intervention should be performed in a manner that does not compromise treatment blinding and ensures product integrity. The existing protocol requirements for product accountability remain unchanged, including verification of participant's receipt of study supplies.
- When delivering supplies to a location other than the study site (for example, participant's home), the investigator, sponsor, or both should ensure oversight of the shipping process to ensure accountability and product quality (that is, storage conditions maintained and intact packaging upon receipt).
- Instructions may be provided to the participant or designee on the final disposition of any unused or completed study supplies.

If study intervention will be administered to the participant at an alternate location, this additional requirement must be met:

- Only authorized study personnel may supply, prepare or administer study intervention.

Adjustments to visit windows

Whenever possible and safe to do so, as determined by the investigator's discretion, participants should complete the usual SoA. To maximize the possibility that the specified visits can be conducted as on-site visits, the windows for visits may be adjusted, upon further guidance from the sponsor. This minimizes missing data and preserves the intended conduct of the study. The dosing schedule should remain consistent with what is specified in the SoA.

This table describes the allowed adjustments to visit windows.

Visit Number	**Tolerance**
Visit 1	Completed within ≤90 days after availability of screening biomarker result; CDR must be performed within 90 days before the first dose
Visit 2	Visit 2 concludes no later than 90 days after the participant's Visit 1 CDR assessment
Visit 3 through 10, and Visits 101 through 109	Visit interval tolerance (days) ±14 days

Abbreviation: CDR = Clinical Dementia Rating.

For participants whose visits have extended windows, additional study intervention may need to be provided to avoid interruption and maintain overall integrity of the study.

Documentation

Changes to study conduct will be documented

Sites will identify and document the details of how participants, visit types, and conducted activities were affected by exceptional circumstances. Dispensing/shipment records of study intervention and relevant communications, including delegation, should be filed with site study records.

Source documents at alternate locations

Source documents generated at a location other than the study site should be part of the investigator's source documentation and should be transferred to the site in a secure and timely manner.

10.10. Appendix 10: Abbreviations and Definitions

Term	Definition
Aβ	amyloid-β
abuse	use of a study intervention for recreational purposes or to maintain an addiction or dependence
AD	Alzheimer's disease
ADA	anti-drug antibodies
ADCS	Alzheimer's Disease Cooperative Study
ADCS-ADL-PI	ADCS Activities of Daily Living – Prevention Instrument
ADE	adverse device effect
AE	adverse event
AESI	adverse event of special interest
ALP	alkaline phosphatase
ALT	alanine aminotransferase
APOE	apolipoprotein E
ARIA	amyloid-related imaging abnormalities
ARIA-E	amyloid-related imaging abnormality-edema/effusion
ARIA-H	amyloid-related imaging abnormality-microhemorrhages and hemosiderin deposits and/or cortical superficial siderosis
AST	alanine aminotransferase
authorized AxMP	*Applicable to the EU only*: a medicinal product authorized in accordance with Regulation (EC) No 726/2004, or in any Member State concerned in accordance with Directive 2001/83/EC, irrespective of changes to the labelling of the medicinal product, which is used as an auxiliary medicinal product
authorized IMP	*Applicable to the EU only*: a medicinal product authorized in accordance with Regulation (EC) No 726/2004 or in any Member State concerned in accordance with Directive 2001/83/EC, irrespective of changes to the labelling of the medicinal product, which is used as an investigational medicinal product

Term	Definition
AxMP	auxiliary medicinal product. See also NIMP. A medicinal product used for the needs of a clinical trial as described in the protocol, but not as an investigational medicinal product. Examples include rescue medication, challenge agents, agents to assess endpoints in the clinical trial, or background treatment. AxMP does not include investigational medicinal product (IMP) or concomitant medications. Concomitant medications are medications unrelated to the clinical trial and not relevant for the design of the clinical trial
blinding	A double-blind study is one in which neither the participant nor any of the investigator or sponsor staff who are involved in the treatment or clinical evaluation of the subjects are aware of the treatment received.
CDR	Clinical Dementia Rating
CDR-GS	Clinical Dementia Rating - Global Score
CDR-SB	Clinical Dementia Rating - Sum of Boxes
CFI	Cognitive Function Index
CI	confidence interval
CFR	Code of Federal Regulations
complaint	A complaint is any written, electronic, or oral communication that alleges deficiencies related to the identity, quality, purity, durability, reliability, safety or effectiveness, or performance of a drug or drug delivery system.
compliance	Adherence to all study-related requirements, GCP requirements, and applicable regulatory requirements
CPAL	Continuous Paired Associate Learning
CRF	case report form; a printed, optical, or electronic document designed to record all of the protocol-required information to be reported to the sponsor for each trial participant.
C-SSRS	Columbia-Suicide Severity Rating Scale
CSR	clinical study report
CT	computed tomography
CTA	Clinical Trial Agreement
DMC	data monitoring committee. A data monitoring committee, or data monitoring board (DMB) is a group of independent scientists who are appointed to monitor the safety and scientific integrity of a human research intervention, and to make recommendations to the sponsor regarding the stopping of a study for efficacy, or for harm, or for futility. The composition of the committee is dependent upon the scientific skills and knowledge required for monitoring the particular study.

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Term	Definition
DSST	Digit Symbol Substitution Test
ECG	Electrocardiogram
eCOA	electronic clinical outcome assessment
EDC	electronic data capture
enroll	The act of assigning a participant to a treatment. Participants who are enrolled in the study are those who have been assigned to a treatment. For this study, enrollment occurs by means of randomization to treatment.
enter	Participants entered into a study are those who sign the informed consent form directly or through their legally acceptable representatives.
ERB	ethics (or ethical) review board. An independent body constituted of medical, scientific, and nonscientific members, whose responsibility is to ensure the protection of the rights, safety, and well-being of human participants involved in a trial by, among other things, reviewing, approving, and providing continuing review of trials, of protocols and amendments, and of the methods and material to be used in obtaining and documenting informed consent of the trial participants
ETa	early termination (discontinuation) visit during Double-Blind Treatment Period or Double-Blind Observation Period
ETb	early termination (discontinuation) visit during Open-Label Extension Period
EU	European Union
EVa	Extra Visit, first, third, fifth, etc.
EVb	Extra Visit, second, fourth, sixth, etc.
FAQ	Functional Activities Questionnaire
GCP	good clinical practice
IB	Investigator's Brochure
ICF	informed consent form
ICH	International Council for Harmonisation
iDSSTm	International Daily Symbol Substitution Test-Medicines
IEC	independent ethics committee; see "ERB"
IgA	immunoglobulin A
IgG	immunoglobulin G

Term	Definition
IMP	Investigational Medicinal Product (see also "investigational product") A medicinal product which is being tested or used as a reference, including as a placebo, in a clinical trial.
informed consent	A process by which a participant voluntarily confirms their willingness to participate in a particular study, after having been informed of all aspects of the study that are relevant to the participant's decision to participate. Informed consent is documented by means of a written, signed and dated informed consent form. As used in this protocol, the term "informed consent" includes all consent given by the participant or their legal representatives (or equivalent in accordance with local law) and by their study partners.
INR	international normalized ratio
interim analysis	An interim analysis is an analysis of clinical study data, separated into treatment groups, that is conducted before the final reporting database is created/locked.
IP	investigational product. A pharmaceutical form of an active ingredient or placebo being tested or used as a reference in a clinical trial, including products already on the market when used or assembled (formulated or packaged) in a way different from the authorized form, or marketed products used for an unauthorized indication, or marketed products used to gain further information about the authorized form. See also "IMP."
IRB	institutional review board; see "ERB"
ISO	International Organization for Standardization
ISLT	International Shopping List Test
ITT	intention to treat: The principle that asserts that the effect of a treatment policy can be best assessed by evaluating on the basis of the intention to treat a participant (that is, the planned treatment regimen) rather than the actual treatment given. It has the consequence that participant allocated to a treatment group should be followed up, assessed, and analyzed as members of that group irrespective of their compliance to the planned course of treatment.
IWRS	interactive web-response system
legal representative, legally acceptable representative, authorized representative	As used in this protocol, the terms "legal representative," "legally acceptable representative," and "legally authorized representative" include equivalent representative, proxy, or surrogate in accordance with local law, if applicable.
mAb	monoclonal antibody
MBI-C	Mild Behavioral Impairment Checklist
MCI	mild cognitive impairment

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Term	Definition
medication error	Errors in the prescribing, dispensing, or administration of a study intervention, regardless of whether or not the medication is administered to the participant or the error leads to an AE. Medication error generally involve a failure to uphold 1 or more of the 5 "rights" of medication use: the right participant, the right drug, the right dose, right route, at the right time. In addition to the core 5 rights, the following may also represent medication errors: ● dose omission associated with an AE or a product complaint ● dispensing or use of expired medication ● use of medication past the recommended in-use date ● dispensing or use of an improperly stored medication ● use of an adulterated dosage form or administration technique inconsistent with the medication's labeling (for example, Summary of Product Characteristics, IB, local label, protocol), or ● shared use of cartridges, prefilled pens, or both.
misuse	Use of a study intervention for self-treatment that either is inconsistent with the prescribed dosing regimen, indication, or both, or is obtained without a prescription
MMSE	Mini–Mental State Examination
MoCA	Montreal Cognitive Assessment
MRI	magnetic resonance imaging
N3pG Aβ	pyroglutamate modification of the third amino acid of amyloid-β peptide
NIMP	Non-investigational Medicinal Product See "AxMP" A medicinal product used for the needs of a clinical trial as described in the protocol, but not as an investigational medicinal product. Examples include rescue medication, challenge agents, agents to assess endpoints in the clinical trial, or background treatment.
OLE	open-label extension
participant	Equivalent to CDISC term "subject": an individual who participates in a clinical trial, either as recipient of an investigational medicinal product or as a control
PC	product complaint
PK/PD	pharmacokinetics/pharmacodynamics
PT-INR	prothrombin time-international normalized ratio
P-tau	phosphorylated tau
Q4W	every 4 weeks
Q8W	every 8 weeks
QDRS	Quick Dementia Rating System

Term	Definition
QOL-AD	Quality of Life in Alzheimer's Disease
QTL	quality tolerance limit
RUI-B	Resource Utilization Inventory – Brief
SAC	statistical analysis center
SAE	serious adverse event
SAP	statistical analysis plan
SC	subcutaneous
screen	The act of determining if an individual meets minimum requirements to become part of a pool of potential candidates for participation in a clinical study.
SIB	suicidal ideation and behavior
SoA	schedule of activities (Section 1.3)
SUSAR	suspected unexpected serious adverse reactions Refers to an adverse event that occurs in a clinical trial participant, which is assessed by the sponsor and or study investigator as being unexpected, serious and as having a reasonable possibility of a causal relationship with the study intervention.
TBL	total bilirubin; total bilirubin level
TE	treatment-emergent
TE ADA	treatment-emergent anti-drug antibodies
TEAE	Treatment-emergent adverse event: An untoward medical occurrence that emerges during a defined treatment period, having been absent pretreatment, or worsens relative to the pretreatment state, and does not necessarily have to have a causal relationship with this treatment.
TPO	third-party organization. An individual or organization (commercial, academic, or other) contracted by the sponsor to provide services and/or deliverables related to medical research
ULN	upper limit of normal
ZBI	Zarit Burden Interview

Approved on 29 Aug 2025 GMT

10.11. Appendix 11: Protocol Amendment History

Amendment [b]: (11-July-2025)

This amendment is considered to be substantial.

The amendment is considered to be substantial because it is likely to have a significant impact on the reliability and robustness of the data generated in the clinical study.

Overall rationale for the amendment

The main purpose of this amendment is to increase the sample size

Additional changes and a brief rationale are provided in the table below.

Section # and Name	Description of Change	Brief Rationale
1.1. Synopsis 3. Objectives, Endpoints, and Estimands	Removed "CBB measures" in the secondary endpoint of other measures of clinical progression	To align with the updates made to the SoA
1.1. Synopsis 3. Objectives, Endpoints, and Estimands	Removed "and body weight measurements" from the secondary endpoint of safety	Due to non-standardized body weight measurement collection.
1.1. Synopsis	Revised the third paragraph in subsection "Brief Summary" to clarify that throughout the study all visits will have remote components	To provide clarity.
1.3. Schedule of Activities (SoA)	Revised the text above the table to clarify that throughout the study all visits will have remote components	
1.1. Synopsis 9.5. Sample Size Determination	Updated the total number of participants as up to approximately 1400	To more appropriately reflect the number of participants planned for enrollment in Study LAKI
1.3.1. Prescreening Period	Updated comments of Demographics to include years of education	To provide clarity
	Updated comments of *APOE* sample collection to include reference to Section 8.6.1.	
1.3.2. Screening Period	Updated comments on Clinical Dementia Rating (CDR) to read: "Remotely administered by central rater. Should be performed within 63 days before the first dose."	To provide flexibility.
1.3.3 Double-Blind Treatment Period	Added "Weight" at Visit 8	To collect longitudinal weight.
	Removed Cogstate Brief Battery (CBB)	To reduce participant burden.

Section # and Name	Description of Change	Brief Rationale
1.3.3 Double-Blind Treatment Period 1.3.5. Open-Label Extension Period and Visit 801	Updated MRI requirements for temporary discontinuations in dosing.	To update safety monitoring requirements.
1.3.3 Double-Blind Treatment Period 1.3.5. Open-Label Extension Period and Visit 801	Updated comments of row "PK sample – predose and postdose" to include recommendation on collection of postdose samples at least 30 minutes after dosing	To provide clarity.
1.3.4. Double-Blind Observation Period	Added a new row "Weight" at Visit 12, Visit 14, EVa, and ETa	To collect longitudinal weight.
	Removed Cogstate Brief Battery (CBB)	To reduce participant burden.
4.1. Overall Design	Updated the sixth bullet to include details on the dosing sequence Updated the seventh bullet to read: "placebo, given at matching time points (during the double-blind treatment period)." Added "If a dose is withheld for any reason, participants should receive doses in the sequence outlined above. See Section 7.1.2 for additional information on temporary discontinuation."	To provide clarity on dosing sequence to be followed during the study
5. Study Population	Updated subsection "Enrollment limits based on CDR scores" to clarify that enrollment of individuals with CDR-GS >0 may be restricted at the discretion of the sponsor up to approximately one third of the enrolled participants	To provide flexibility in enrollment.
5.1. Exclusion Criteria	Updated criterion #14 to include "are, in the judgment of the investigator, actively suicidal and therefore deemed to be at significant risk for suicide"	To include an inadvertent omission
6.1. Study Interventions Administered	Added details on the dosing sequence Added "If a dose is withheld for any reason, participants should receive doses in the sequence outlined above. See Section 7.1.2 for additional information on temporary discontinuation."	To provide clarification.

Section # and Name	Description of Change	Brief Rationale
	Added a new subsection "Dosing intervals" to include details on dosing intervals for SC Q8W dosing and SC Q4W dosing	To provide clarification.
7.1.2. Temporary Discontinuation (Interruption of Study Intervention)	Revised the second paragraph to include details on steps to be followed during resumption of dosing regardless of the reason	To update safety monitoring requirements.
7.1.2.1. Due to ARIA	Updated subsection "Criteria for temporary discontinuation of study intervention" to include situations in which a participant should temporarily discontinue study intervention Updated subsection "Reinitiating study intervention" to include reference to Section 7.1.2 for more information regarding reinitiating study intervention following resolution of non-serious ARIA-E, stabilization of non-serious ARIA-H, and the resolution of any associated symptoms	To update safety monitoring requirements.
7.1.2.2. Due to Reasons Other Than ARIA	Updated section to include reference to Section 7.1.2 for more information regarding reinitiating study intervention	To update safety monitoring requirements.
8.1. Efficacy Assessments	Updated section to clarify that rater- and self-administered assessments can be conducted either on the same day or on separate days, provided that the rater-administered session is completed first. The self-administered session should occur within 7 days after the rater-administered session if done on separate days	To provide flexibility
8.1.7. Cogstate Brief Battery	Removed the section	To align with changes made in the SoA
8.2.6. Hepatic Safety Monitoring, Evaluation, and Criteria for Study Drug Interruption or Discontinuation	Updated table footnote "a" in both tables	To update the hepatic safety monitoring

Section # and Name	Description of Change	Brief Rationale
8.2.6.1. Close Hepatic Monitoring	Updated table footnote "a" Updated the second bullet in the actions included in close hepatic monitoring to clarify the frequency of monitoring if the participant's clinical condition and laboratory results stabilize	
8.2.6.2. Comprehensive Hepatic Evaluation 8.2.6.3. Study Drug Interruption or Discontinuation	Updated table footnote "b"	
8.2.7.1. C-SSRS	Added "For this study, the full version of the C-SSRS will be used."	To provide clarity.
8.2.8. Magnetic Resonance Imaging	Updated section to include that the unscheduled MRI CRF should be completed to provide rationale for all unscheduled MRIs Revised the last paragraph to clarify that the decision to administer dose should be based on the central read; however, the local read may be used for dosing decisions if the central read is still pending	To provide instructions on CRF completion To provide clarity
8.3.3.1. Amyloid-Related Imaging Abnormalities (ARIA-E and ARIA-H) and Macrohemorrhages	Updated the fourth paragraph to read: "In the event of a macrohemorrhage or an SAE associated with treatment-emergent ARIA, study intervention should be permanently discontinued (Section 7.1), and the participant should be monitored with unscheduled MRIs at least every 4 to 6 weeks until stable."	To provide clarity.
8.6.1. Apolipoprotein E Genotyping	Updated section to include language on *APOE* genotyping and how participants can learn about their results	To provide clarity
9.1. Statistical Hypotheses	Removed "CBB measures"	For editorial consistency

Section # and Name	Description of Change	Brief Rationale
9.5. Sample Size Determination	Updated the second paragraph as follows: • Updated the sample size from 1200 to 1400. • Revised the annualized placebo event rate from "12% to 15%" to "8% to 10%". • Clarified that the hazard ratio assumption was based on the treatment effect observed with other amyloid targeting therapies in early symptomatic AD population	To clarify the sample size adjustment based on updated annualized placebo event rate.
10.5. Appendix 5: Liver Safety: Suggested Actions and Follow-up Assessments	Updated the text above the table to clarify that if testing is not available based on local requirements, this will not be considered a protocol deviation, and the sponsor's designated medical monitor should be consulted Added "(total or IgG)" to the row "HCV antibody" Removed footnotes from both the tables Moved HDV testing to the list of tests to be performed by the investigator-designated laboratory Added the following tests to the list of tests to be performed by the investigator-designated laboratory Iron Ferritin Total Iron Binding Capacity or Transferrin Saturation	To update the hepatic safety monitoring
10.7. Optional APOE Disclosure	Deleted text regarding APOE genotype results and added reference to Section 8.6.1.	To remove redundant text based on the updates made in Section 8.6.1.
10.10. Abbreviations and Definitions	Removed "CBB"	Editorial consistency
10.11 Protocol Amendment History	Added amendment(a) rationale and summary of changes	To update the amendment history

Section # and Name	Description of Change	Brief Rationale
11. References	Removed the following citations Darby et al. (2002) Lim et al. (2014) Lim et al. (2021) Lim et al. (2015) Maruff et al. (2009)	Editorial consistency
Throughout the document	Minor editorial and formatting changes	Editorial consistency

Amendment [a]: (04-December-2024)

The amendment is considered to be substantial.

Overall rationale for the amendment

The main purpose of this amendment is to address regulatory feedback.

Section # and Name	Description of Change	Brief Rationale
1.1. Synopsis	In the subsection about Study Population, added text defining early AD for this study	To provide clarity
	In the subsection about Number of Participants, added the wording "Up to"	To provide clarity
1.3.1. Prescreening Period	In the row for "*APOE* sample collection," removed "only" and added a statement about alternative use of stored samples to determine eligibility for future clinical trials	To ease operations and to reduce burden for individuals interested in participating in future clinical trials
	In the row for "Plasma phosphorylated tau (P-tau)" sample collection, added a statement about alternative use of stored samples to determine eligibility for future clinical trials	To reduce burden for individuals interested in participating in future clinical trials
1.3.2. Screening Period	In the row for "Informed consent of study partner," added "X" in Visit 1 column	To provide clarity and to correct an inadvertent omission
1.3.3. Double-Blind Treatment Period	In the row for "Clinical outcome assessments," changed "must" to "should" in the statement about administration of clinical outcome assessments	To provide flexibility and to ease operations
	In the row for "MRI," removed "X" from Week 32 (Visit 6) and Week 52 (Visit 8) columns, and added "X" in Week 40 (Visit 7) column; corrected Comment field	To optimize the MRI schedule to reduce participant burden and ensure appropriate safety monitoring during dose titration
	In the row for "Dispense study intervention to participant for Q4W dosing," added "X" in the Week 52 (Visit 8) column; also added a statement allowing dispensing at alternative timepoints if study drug was withheld	To provide clarity and to ease operations

Section # and Name	Description of Change	Brief Rationale
1.3.4. Double-Blind Observation Period	In the row for "C-SSRS Since Last Assessed (Full Version)," revised text in the Comment field, stating that C-SSRS will be administered by trained site staff instead of a central rater	To ease operations
	In the row for "Clinical outcome assessments," changed "must" to "should" in the statement about administration of clinical outcome assessments	To provide flexibility and to ease operations
	In the row for "MRI," deleted "X" in Week 78 (Visit 11) column	To optimize the MRI schedule to reduce participant burden
	In the row for "No dosing in this study period," added text in the Comment field explaining that although there is no planned dosing regimen for this period, participants who were temporarily discontinued from study intervention in the previous period might continue to be dosed in this period, as described in Section 7.1.2	To provide clarity
1.3.5. Open-Label Extension Period and Visit 801	In the row for "MRI," deleted "X" in Week 32 (Visit 105), Week 52 (Visit 107), and Week 78 (Visit 110) columns; added "X" in Week 40 (Visit 106) column; corrected Comment field accordingly	To optimize the MRI schedule to reduce participant burden and ensure appropriate safety monitoring during dose titration.
	In the row for "Dispense study intervention to participant for Q4W dosing," added "X" in Week 52 (Visit 107) column; also added a statement allowing dispensing at alternative timepoints if study drug was withheld	To provide clarity and to ease operations
2.2. Background	Revised statements about the investigational status of donanemab, stating that donanemab and lecanemab have been approved by several countries	To provide more current information
3. Objectives, Endpoints, and Estimands	In the "Intercurrent events" text, added "while" before the phrase "on-treatment" in the first sub-bulleted text	To provide clarity
	In the "Intercurrent events" text, added a third sub-bullet about intercurrent events of death	To address regulatory feedback
	Added a sentence with a cross-reference to Section 9.3.2. Added a sentence about sensitivity analysis using an estimand strategy for use of prohibited medications as an intercurrent event	To provide clarity

Section # and Name	Description of Change	Brief Rationale
4.1. Overall Design	Added a statement explaining that participants assigned to remternetug at randomization will complete the full course of dosing after the target number of primary outcome events has been observed	To address regulatory feedback
4.2. Scientific Rationale for Study Design	In the paragraph about study population, revised the summary and added citations to elaborate on the definition of the population for this study	To address regulatory feedback and to be more concise
	Replaced "Alzheimer's Association 2023" with "Jack et al. 2024"	To include a more current citation
4.4. End of Study Definition	Added a definition of end-of-study according to "end of treatment" while retaining a definition of end-of-study for the primary outcome	To address regulatory feedback
5. Study Population	Added an unnumbered subheading called "Screening extensions for certain demographics" and added a sentence to enable extended screening for certain geographies	To support adequate enrollment in countries where a particular enrollment target must be met for registration purposes
	Under the unnumbered subheading called "Enrollment limits based on CDR scores," added the word "up" in the phrase "restricted up to approximately"	To improve clarity
5.2. Exclusion Criteria	In Criterion [16], removed the words "at screening/baseline" and inserted "prior to randomization"	To improve clarity
	In Criterion [19], the threshold for cerebral microhemorrhages was lowered	To mitigate potential risk of ARIA
	In Criterion [20], revised text to exclude participants with any history of prior treatment with passive anti-amyloid immunotherapy	To more appropriately reflect medication use in an early AD patient population
	In Criterion [28], added that participants are excluded if their study partners are site personnel involved in this study	To improve clarity and reduce potential bias in assessments
5.2.1. Criteria for Open-Label Extension Period	Added a new subsection heading; within that subsection, specified certain inclusion and exclusion criteria for the open-label extension period	To address regulatory feedback and to improve clarity
	In Criterion [30], revised the language to exclude participants from participating in the open-label extension who have any ARIA-E, a defined threshold of cerebral microhemorrhage, any macrohemorrhage, or severe white matter disease as well as any superficial siderosis	To address regulatory feedback

Section # and Name	Description of Change	Brief Rationale
8.1.1. Clinical Dementia Rating (CDR)	Deleted statement about CDR being administered by the same rater from visit to visit	To provide flexibility and to ease operations
8.2.7.1. C-SSRS	Added a sentence stating that C-SSRS data will be collected via an eCOA tablet	To improve clarity
8.3.3. Adverse Events of Special Interest	In the bulleted list, added the word "hemorrhage" after "cerebral" and made "macrohemorrhage" into a parenthetical phrase	To improve clarity
8.3.3.1. Amyloid-Related Imaging Abnormalities (ARIA-E and ARIA-H) and Macrohemorrhages	In the subsection title, added the phrase "and Macrohemorrhages"; in the text, added sentences about macrohemorrhages along with a cross reference to Section 7.1	To improve clarity and to be consistent with existing text in Sections 2.3.1 and 7.1
9.3.1. General Considerations	Deleted sentences to eliminate ambiguity regarding the use of ITT principle	To address regulatory feedback
9.3.2. Primary Endpoint Analysis	Added a statement about following ITT principle for primary analysis	To address regulatory feedback
9.4. Interim Analyses	Added a statement that the protocol will be amended prior to interim analyses of early efficacy	To address regulatory feedback
9.5. Sample Size Determination	Provided literature citations supporting the hazard ratio	To support the sample size and to address regulatory feedback
10.4. Appendix 4: Contraceptive and Barrier Guidance	In the subsection providing a definition of the term "Male," added the word "bilateral" before the word "orchidectomy"	To address regulatory feedback
10.6.2. Japan	Revised text to allow the initial informed consent to be collected either remotely or virtually or in person	To enable flexibility in the methods for collection of informed consent
	Deleted text about collection of height and weight remotely or at site	To remove an unnecessary clarification
10.6.3. South Korea	Added a new subheading and a statement about contraception use by IOCBP if IOCBP are inadvertently enrolled	To address regulatory feedback
11. References	Removed Alzheimer's Association 2023 and Leqembi package insert. Added articles by Dubois et al. 2021, Jack et al. 2024, Malek-Ahmadi 2016, Wood 2016, and Yu et al. 2024	To include references cited in other sections
Throughout	Minor spacing, punctuation, and formatting adjustments that do not change meaning of the text	Editorial consistency

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